

A Voyage into Trauma

The Soma-Field Theory of Emotional Life

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A Voyage into Trauma

The Soma-Field Theory of Emotional Life

Alistair Johnson

2026

For everyone who was told their body was overreacting. It wasn't. It was solving the right problem.

Preface: The T's

This book began as a physics paper.

It ended as a map.

The paper was called the Soma-Field Model, and it was written in the language of physicists: Hamiltonians, propagators, coupling matrices, Wick rotations. It was precise. It was, I think, correct. And it was almost entirely unreadable to the people it was most about.

This book is the translation.

There are four T's running through what follows, and they are not accidental. The first is **Trauma** — the subject. The second is **Threshold** — a specific parameter in the model, written T , that marks the boundary between what becomes conscious and what stays body. The third is **Time** — in particular, developmental time, the age at which a modification occurred, which turns out to matter enormously. The fourth is **Transformation** — not recovery, not a return, but a going forward into a wider landscape.

There is a fifth T that I notice only in retrospect: **Trance** — in two senses simultaneously. *A Voyage into Trance* is a 1995 Goa trance compilation by Paul Oakenfold; the title of this book is borrowed from it. A trance state is, in the language of the Soma-Field Model, a phase transition of the emotional field — a threshold crossing guided by sound and rhythm rather than threat. The trance state produced by extended rhythmic music at 140 BPM and the freeze response of a traumatised nervous system are not the same experience. They are governed by the same mathematics: the

same threshold crossings, the same phase transitions, the same field dynamics. The T's of that album and the T's of this book are the same T's.

I should tell you what happened in 1968 before we go any further, because it is the reason this model exists and the reason I am the one writing it. I was approximately eighteen months old. I developed septic arthritis in my left hip — a bacterial infection of the joint that, without treatment, destroys the socket entirely. It was treated, and I recovered. The treatment involved three months in hospital under what were then called “no-touch protocols”: the infection risk was judged to require isolation from physical contact. Three months is a long time at eighteen months. The body learns quickly at that age, and what mine learned — in the absence of any other available hypothesis — was that the world was a place where pain arrived without warning and comfort did not follow.

That is not a complaint. The clinicians saved the joint. But the learning happened, and it happened before language, before narrative memory, before the self that can tell the story was formed. The modification was not added to an existing structure. It was the structure.

This book is, among other things, an attempt to say that formally — to give that experience a mathematical description precise enough to make predictions, to inform clinical practice, and to explain why the goal is not to return to a self that never existed, but to build forward into one that can.

I should also tell you that I promised, years ago, to write a book about a campsite in the Glarus Alps of Switzerland — a valley with parabolic limestone walls that make sound oscillate like a natural resonator, adjacent to one of the world's great tectonic structures. The book about the campsite and the book about the soma-field found each other. The Interlude between Part II and Part III is where they meet.

The physics is real. The equations correspond to something. And the voyage is the proof of it.

Alistair Johnson May 2026

How to Read This Book

This book is written for three kinds of reader, and you can navigate it differently depending on which you are.

If you are new to all of this — no physics background, no clinical background, just a body that has been confusing you — read Part I first. Chapters 1 through 3 are written for you. The mathematics in those chapters is kept to a minimum; the ideas are introduced through physical intuition and lived experience. When equations do appear, they are explained in words immediately. Nothing is assumed except curiosity.

If you are a mental health professional who wants to understand what this model adds to your existing framework — you can begin with the Part I overview and then move directly to Parts III and IV. The Going Deeper boxes throughout the book are written for you. The appendices contain the full mathematics as it appears in the academic paper.

If you are a physicist, mathematician, or computationalist who has arrived here by accident or curiosity — you will recognise the Hamiltonian formulation immediately. The novel content for you is in Chapters 6, 7, and Appendix A. The Lean 4 type sketches in Appendix B may be of particular interest; they are incomplete proofs, marked with **sorry** where the hard work remains, and they represent a research programme.

A note on boxes. Throughout the book you will find four types:

LEARNING OBJECTIVES — what this chapter sets out to establish, listed at the start.

AUTHOR’S NOTE — personal first-person sections. The research and the life it emerged from are not separate, and I have not pretended otherwise.

GOING DEEPER — technical sections with more mathematics or formal detail. They can be skipped without losing the main argument. They can also be read first if that is your preference.

KEY TERMS — precise definitions for terms that carry specific meaning in this model. A full glossary is in Appendix D.

PART I: THE BODY KNOWS

Chapter 1: What the Body Remembers

"The body keeps the score."

– Bessel van der Kolk, 2014

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- Why the body responds to safety and danger independently of what the conscious mind believes
 - What the freeze response is and why it exists
 - The difference between a body that is “overreacting” and a body that has learned accurately
 - Why the first step in understanding trauma is understanding that survival responses are not mistakes
-

1.1 The Waiting Room

Picture a waiting room. You are sitting in a chair, waiting for a routine appointment. Nothing unusual is happening. The lighting is fluorescent. There is a plant in the corner that needs watering. Someone across the room is reading a magazine with the

particular focused patience of someone who is not, in fact, reading a magazine.

For most people in this waiting room, the body is doing nothing remarkable. Heart rate is steady. Breathing is even. The background hum of vigilance that keeps us alive is running at its ordinary level.

For some people, this waiting room is already an event. Heart rate has been elevated since the appointment was booked. Breathing is shallower than usual. There is a low-frequency alertness — a readiness — that is using energy, keeping muscles slightly contracted, keeping the hearing slightly sharpened, keeping the eyes moving. The mind may know that this is a routine appointment. The body has reached a different conclusion and is acting on it.

This is not anxiety in the clinical sense, though it may be diagnosed as such. It is not a failure of rational thinking. It is a body doing exactly what it learned to do — and doing it accurately, given what it knows.

The question this book asks is: what does the body know, and how did it learn it, and what does it mean to change that learning?

1.2 What Trauma Is (and Is Not)

The word *trauma* is used in many ways. In this book, it has a specific meaning.

Trauma is a **permanent modification of the nervous system's prediction model** in response to an experience that exceeded the system's capacity to process and integrate at the time of the experience.

Notice what this definition does and does not say.

It does **not** say that trauma is a weakness. A bridge that bends under a load it was not designed for is not a weak bridge — it is a bridge responding appropriately to a

force that exceeds its design specifications.

It does **not** say that trauma is a mental event. The modification happens at the level of the nervous system — in the way sensory signals are filtered, in the way the body prepares for action, in the way energy is distributed across physiological systems. These are physical processes.

It does **not** say that the traumatic event needs to be dramatic. A three-month absence of contact comfort at eighteen months of age is not a bomb going off. It is, by most conventional standards, a minor medical intervention. What matters is the match between the experience and the system's current capacity — and at eighteen months, the nervous system has no framework whatsoever for “temporary separation for medical reasons.”

What trauma **is**: a successful adaptation. The nervous system encountered a situation it could not model, and it updated its model in the direction that maximised survival. If the world is a place where pain arrives without warning and comfort does not follow, then a nervous system set to high alert — always scanning, always slightly contracted, always ready — is a nervous system correctly calibrated to that world. The problem is not that the calibration is wrong. The problem is that the world has changed and the calibration has not.

AUTHOR'S NOTE: The Hospital, 1968

I do not remember it. I was too young for explicit memory to have formed.

What I have are the downstream signals: a body that has always treated routine medical environments as emergencies. A nervous system that identifies “caring professional approaching to help” and responds with the physiology of threat. A skeleton of responses so deeply embedded that they long

predate any narrative I have been able to construct about them.

My developmental age at the time was approximately eighteen months. The modification that happened then was not added to an existing nervous system. It was the nervous system being formed. That is a distinction that will matter a great deal in Chapter 6.

For now: the body knows things that the mind never learned. This book is an attempt to write those things down in a language precise enough to work with.

1.3 The Polyvagal Ladder

In the 1990s, neuroscientist Stephen Porges developed what he called Polyvagal Theory — an account of the autonomic nervous system that begins not with the familiar fight-or-flight response but with the evolutionary history of the structures involved.

The key observation is this: the autonomic nervous system is not a single dial that runs from “calm” to “alarmed.” It is a hierarchy of three systems, each older than the one above it, each more primitive, each mobilised in sequence as the perceived threat increases.

VENTRAL VAGAL STATE		Social engagement branch
Safe, connected, curious		Myelinated vagus nerve
Window of Tolerance		Heart rate regulated
_____ ← most recently evolved		
SYMPATHETIC STATE		Mobilisation branch

	Alert, energised, defensive	Spinal cord pathway	
	Fight or flight	Heart rate elevated	
	—————	← older	

	DORSAL VAGAL STATE	Immobilisation branch	
	Shutdown, collapse, freeze	Unmyelinated vagus nerve	
	Dissociation, numbing	Heart rate dropped	
	—————	← most ancient	
└-----┘			

Figure 1.1. The polyvagal hierarchy. Under conditions of safety, the most evolved system (ventral vagal) governs – enabling social connection, learning, and curiosity. As perceived threat increases, the sympathetic system activates, preparing the body for action. If the threat is overwhelming or escape is impossible, the oldest system (dorsal vagal) takes over: immobilisation, shutdown, disconnection. Trauma often involves the system being stuck at a lower rung long after the original threat has passed.

The critical word in that last sentence is *perceived*. The hierarchy responds to what the body detects as dangerous, not to what the thinking mind judges as dangerous. These are different processes. The thinking mind can be entirely convinced that there is no danger — and the body can, at exactly the same moment, be running the threat response at full intensity. Both are responding to real information. They are just reading different signals.

1.4 The Freeze Response

The freeze response is the least understood of the three states and, for many trauma survivors, the most characteristic.

It is not the absence of a response. It is a full physiological engagement — the body is doing something, and doing it with considerable energy. What it is doing is playing dead.

This is a deeply ancient response. In evolutionary terms, freezing when threatened by a predator is sometimes the optimal move: many predators respond to movement, and a motionless prey item may not register as prey at all. The freeze response in mammals also involves the release of endogenous opioids — nature's way of making the potential experience of being killed slightly less intolerable. This is why dissociation during overwhelming trauma is sometimes described as a kind of mercy.

For humans in modern environments, the freeze response is triggered not by literal predators but by anything the nervous system has learned to classify as equivalent. A raised voice. A medical environment. A particular combination of sensory signals that, at some point in the past, preceded something overwhelming. The body does not distinguish between the original context and the contemporary one. It responds to the signal, not the story.

The result is a person who, in the middle of a conversation or a clinic appointment or an otherwise ordinary moment, suddenly goes quiet and still and seems to be looking at something slightly to the left of wherever they are. They are not being difficult. They are not choosing not to engage. They are playing dead because the body has concluded that this is the appropriate moment to play dead.

1.5 Why This Matters for Treatment

If trauma is a modification of a prediction model — an accurate learning from an overwhelming experience — then the therapeutic question is not *how do we fix the broken response* but *how do we update the prediction model with new information*.

This is a different question, and it has a different answer.

Fixing a broken response implies that the body is malfunctioning. Updating a prediction model implies that the body has been doing its job correctly, and that the job now requires new data.

The Soma-Field Model, which is the subject of this book, provides a mathematical framework for what “updating the prediction model” means precisely: what structures change, what the before and after states look like, and — critically — what kind of change is possible given when the original learning occurred.

That last point is where this model adds something that existing frameworks do not provide, and it is the subject of Chapter 6.

KEY TERMS

Soma — the body as experienced from the inside; the totality of interoceptive (internally sensed) signals.

Polyvagal hierarchy — the three-level autonomic nervous system described by Porges, ordered from most evolutionarily ancient (dorsal vagal) to most recently evolved (ventral vagal).

Window of Tolerance — the range of arousal within which the nervous system can function flexibly, process information, and engage socially. Above the window: hyperarousal (sympathetic). Below the window: hy-

poarousal (dorsal vagal shutdown).

Freeze response — the immobilisation state of the dorsal vagal system; the body's oldest threat response, involving disconnection, stillness, and endogenous opioid release.

CHAPTER SUMMARY

Trauma is a modification of the nervous system's prediction model — a successful adaptation to overwhelming experience, not a failure or weakness. The polyvagal hierarchy describes how three evolutionary layers of the autonomic nervous system respond to perceived threat. The freeze response is the most ancient: immobilisation as survival strategy. For treatment to work, it must address the body's model, not argue with the thinking mind. The Soma-Field Model provides a mathematical language for this.

Chapter 2: A Field of Feeling

"The body is the unconscious mind."

– Candace Pert, 1997

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- What a physical field is and why emotion behaves like one
- Why emotions are body phenomena, not brain phenomena
- What interoception is and why it is fundamental
- The meaning of “the soma-field” as a technical term

2.1 What a Field Is

Imagine the gravitational field of the Earth.

You cannot see it. You cannot touch it. You cannot pick up a handful of gravity and examine it under a microscope. But it is real — as real as anything in physics — and you have felt it every moment of your existence. It is everywhere in space around the Earth. It has a *strength* at every point (stronger close to the Earth, weaker further away). It has a *direction* at every point (towards the centre of the Earth). And it

exerts a force on everything that is in it.

A field, in physics, is precisely this: a quantity that has a value at every point in space. Temperature is a field. The wind is a field (a vector field — direction and magnitude at every point). The electromagnetic field is a field. Quantum fields, which are the foundation of modern physics, are fields.

The key insight of the Soma-Field Model is this: **emotion is a field phenomenon**.

Not a metaphor. A precise claim about how emotional signals distribute themselves in the body, interact with each other, and evolve over time.

2.2 Emotions in the Body

Antonio Damasio, in his somatic marker hypothesis (1994), proposed that emotions are fundamentally body states: that what we call “emotion” is the brain’s representation of a pattern of physiological activation — heartrate, muscle tension, gut movement, hormonal state, skin conductance, respiratory rhythm. We do not feel an emotion and then notice body signals. The body signal *is* the emotion; what the brain does is read it.

This is deeply counterintuitive if you have spent your life inside a culture that treats the mind as the real thing and the body as its vehicle. But the neuroscience supports it consistently. Patients with damage to the parts of the brain that receive and integrate body signals do not make better decisions because they are freed from emotional interference — they make worse decisions, because they have lost access to the somatic markers that tell them which options feel safe and which feel dangerous.

The body is not an obstacle to clear thinking. It is the substrate of it.

Interoception is the technical name for the body’s ability to sense its own internal state: heartbeat, breath, gut sensation, muscle tone, the position of limbs, the temperature

of organs. Interoceptive accuracy — how precisely a person can read their own body signals — varies widely between individuals and is significantly disrupted by trauma.

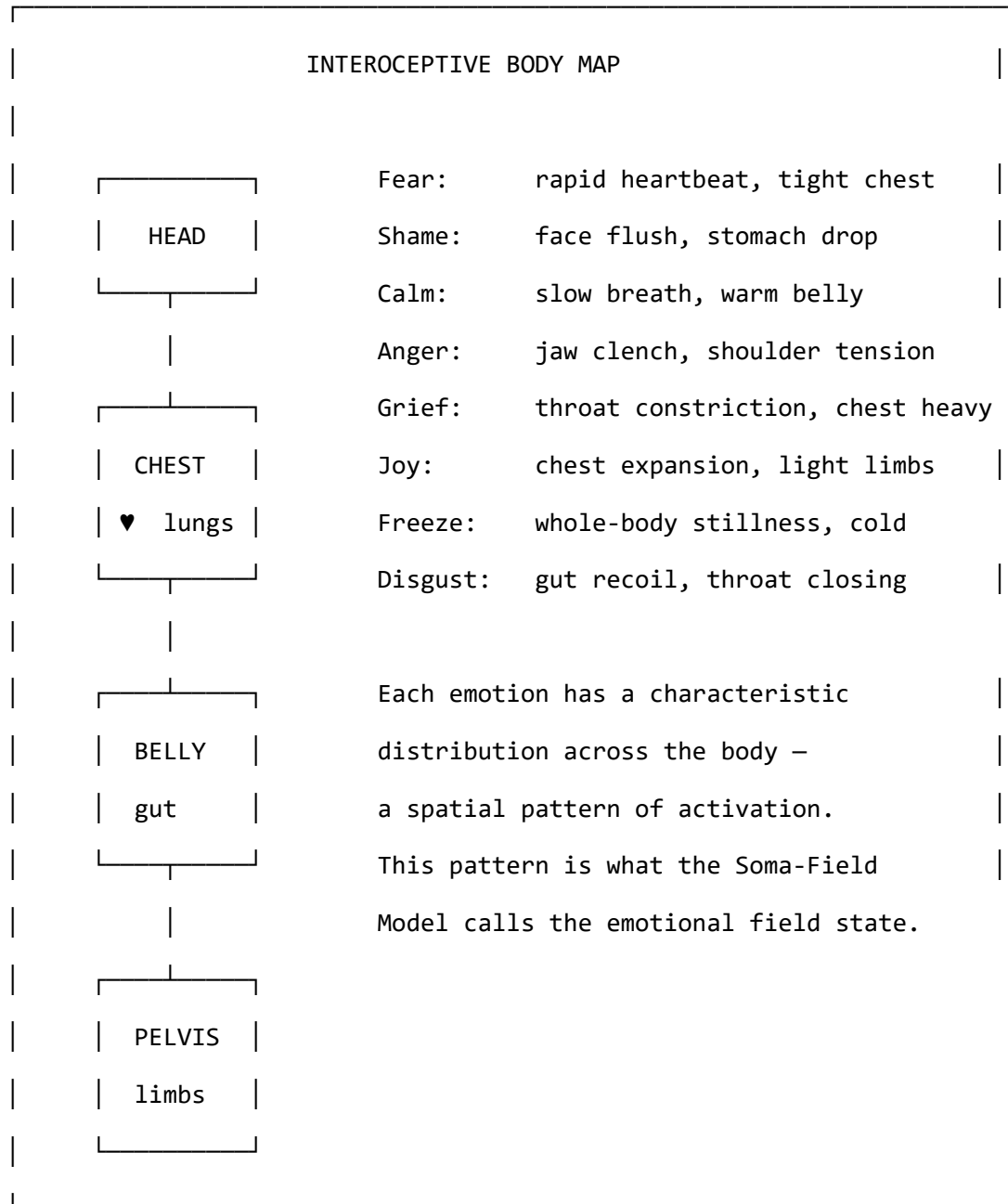


Figure 2.1. The body map of emotional activation. Emotions are not events in the head;

they are distributed patterns of physiological arousal across the body. Research by Nummenmaa et al. (2014) mapped these patterns by asking participants to colour body silhouettes where they felt each emotion. The patterns are consistent across cultures.

GOING DEEPER: The Foot That Isn't There

Pain is not in the foot. It is in the brain's model of the foot.

The clearest proof is phantom limb pain. When a limb is amputated, many patients continue to feel it — and feel it *hurting*. The foot is gone. The pain is real. It wakes people at night, responds to analgesics, and can be agonising for years. What is in pain is the brain's neural map of the foot, which persists in the cortex long after the tissue is gone.

Ramachandran's solution was a mirror box. A mirror placed along the body's midline creates a reflection of the intact hand where the absent hand should be. The patient watches the reflection move. The brain's model updates: *the hand is there, the hand is moving, the hand is fine*. For many patients, the pain decreases or disappears. The model changed. The suffering reduced. Nothing in the body changed at all.

This is not a curiosity. It is the normal condition of all somatic experience. The brain does not receive raw body signals and display them. It maintains a continuous predictive model of the body and generates what you *feel* from that model. The felt body is the predicted body.

For the Soma-Field Model, this is load-bearing. The field $\mathbf{e}(t)$ is not a readout of the physical body. It is the nervous system's model of the body. When the model is updated — by new sensory experience, somatic therapy, or the slow accumulation of safety — what is felt changes. Not because the

body changed. Because the prediction changed.

Therapy does not fix the body. It updates the model.

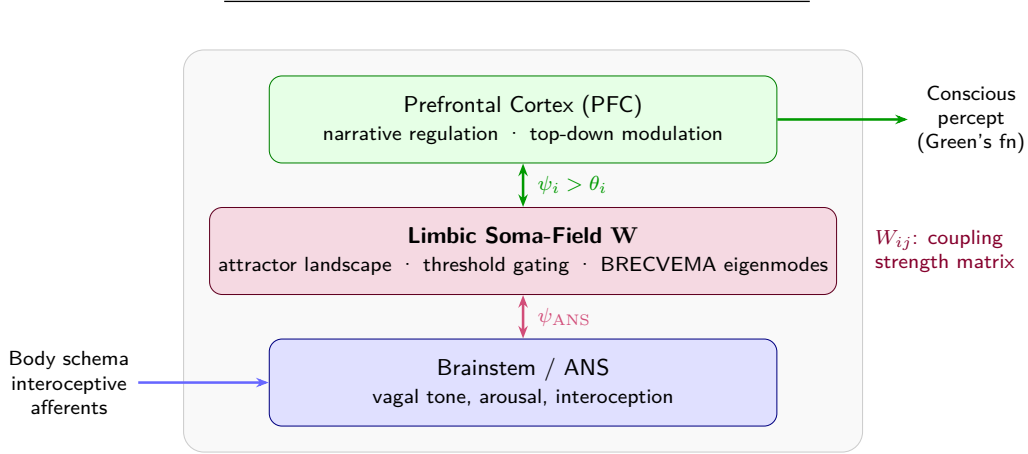


Figure 1: Figure 2.1. The body–brain coupling stack. Interoceptive signals from the body feed into the brainstem and autonomic nervous system, which couples bidirectionally to the limbic soma-field (coupling matrix $\mathbf{W}$). The field gates input to the prefrontal cortex via the threshold ; what crosses becomes conscious percept. *Author's original figure.*

2.3 The Soma-Field: A Technical Definition

In the Soma-Field Model, we represent the body's emotional state as a vector of activation levels across a set of emotional dimensions. Call this vector $\mathbf{e}$:

$$\mathbf{e} = (e_1, e_2, \dots, e_n)$$

Each e_i is a real number representing the activation level of a somatic emotional mode at a given moment: the level of fear-readiness in the body, the level of grief-contraction, the level of social-engagement openness, and so on. The exact labelling of the modes

is secondary to the structure; what matters is that there is a space of such states and a dynamics on that space.

The soma-field is this vector $\mathbf{e}$, evolving in time. It is not a single number (arousal level) or a pair of numbers (valence and arousal). It is a multi-dimensional state that captures the full texture of somatic experience at a given moment.

Three things are immediate from this definition:

1. **The field has a position:** the current emotional state is a point in an n -dimensional space.
2. **The field has dynamics:** it moves through this space over time.
3. **The field has a structure:** some positions are stable (attractors), and the dynamics drives the field toward them.

The next chapter is about that structure.

GOING DEEPER: Quantum Fields and Why They Are Relevant

In quantum field theory (QFT), the fundamental objects are not particles but fields — wave-like disturbances propagating through space. Particles are what you see when a field vibrates at a high enough amplitude to be detected: an electron is a ripple in the electron field, a photon is a ripple in the electromagnetic field.

The soma-field is not a quantum field in the literal sense. Emotional dynamics are classical, not quantum. What the Soma-Field Model borrows from QFT is the *mathematical language*: the same equations that describe how quantum fields couple to each other turn out to describe how emotional modes couple to each other. This is not because emotion is quantum-

mechanical. It is because coupling dynamics — the mathematics of how things that interact shape each other’s behaviour — takes the same form wherever it appears.

This correspondence is the subject of Chapter 7. For now: the QFT connection is a mathematical tool, not a metaphysical claim.

KEY TERMS

Field — a quantity with a value at every point in space (or, in the soma-field context, at every point in the body’s state space).

Interoception — the nervous system’s process of sensing the internal state of the body.

Interoceptive accuracy — the precision with which a person can consciously read their own interoceptive signals.

Soma-field — the vector $\mathbf{e}$ of somatic activation levels across emotional modes; the state of the body’s emotional field at a given moment.

State space — the set of all possible values of $\mathbf{e}$; the arena within which emotional dynamics occurs.

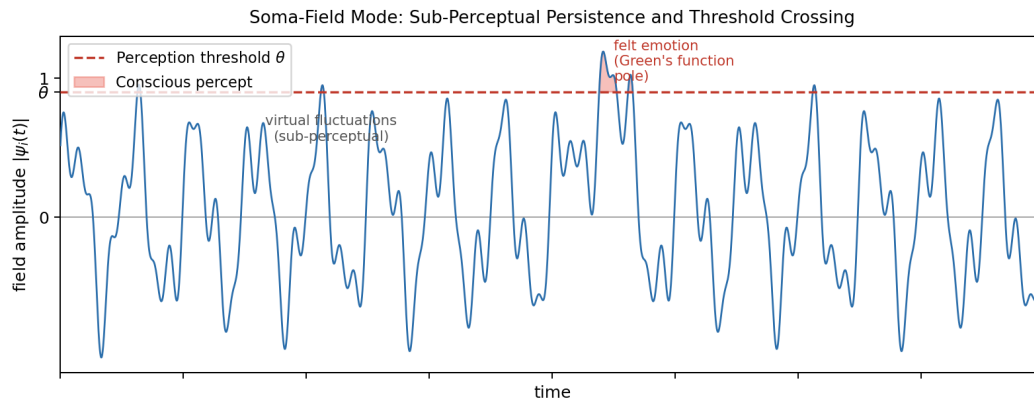


Figure 2: Figure 2.2. The soma-field oscillates continuously. Most of the time its modes remain below the perception threshold T (dashed line) — sub-threshold activity that drives physiology and behaviour invisibly. Only a sufficiently large excitation crosses T into felt experience. Interoceptive training and somatic therapy work, in part, by lowering T . *Author's original figure.*

Chapter 3: The Energy Landscape

"Nature does not create mountains and valleys at random.
They are shaped by the forces beneath them."

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- Why some emotional states are stable and others are transient
- The meaning of “attractor” and “basin of attraction”
- What the Hamiltonian is and why it organises the model
- Why you keep returning to familiar emotional states even when you

don't want to

3.1 Hills and Valleys

Imagine placing a ball on a hilly landscape. If you place it at the bottom of a valley and give it a small push, it rolls away from where you pushed it — and then rolls back. The valley is stable. The bottom of the valley is an *attractor*: the ball is drawn toward it from nearby positions.

If you place the ball at the top of a hill and give it a small push, it rolls away from the hilltop — and keeps going. The hilltop is *unstable*. Small perturbations grow into large departures.

The same geometry applies to emotional states.

Some emotional states are at the bottom of valleys in the body's landscape: they are stable, they are where the system tends to rest, and perturbations that push the body away from them are followed by a return. Other states are at hilltops: they are unstable configurations that the system passes through on its way between valleys.

The crucial question — the question that distinguishes a regulated nervous system from a dysregulated one, and distinguishes one person's landscape from another's — is: where are the valleys? How deep are they? How wide? How many are there?

3.2 Attractors and Basins

An **attractor** is a stable state — a bottom of a valley. A **basin of attraction** is the set of all points from which the system rolls toward a given attractor: the “catchment area” of the valley.

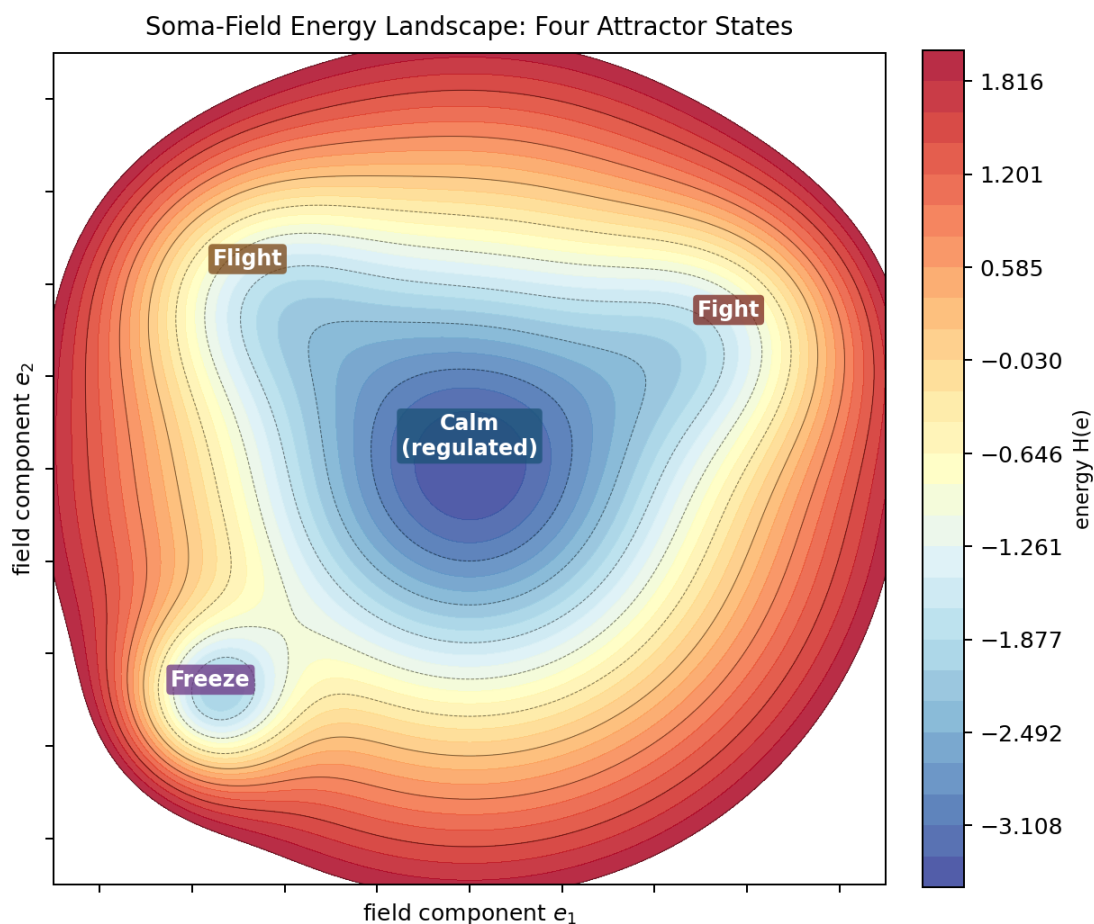


Figure 3: Figure 3.1. The emotional energy landscape (2D contour). Four attractor basins are visible: Calm (wide, deepest — the global minimum of a regulated nervous system), Freeze (narrow and very deep — easy to fall into, hard to leave), Fight and Flight (intermediate depth). The system rolls downhill to the nearest basin; the depth controls escape difficulty and the width controls resilience to perturbation. *Author's original figure.*

For a regulated nervous system, the primary attractor is some version of calm social engagement — the ventral vagal state of Polyvagal Theory. The basin is wide: a large range of perturbations (emotions, sensations, social situations) all resolve back to this resting state. The system is resilient.

For a trauma-modified nervous system, the landscape has changed. A second attractor — hypervigilance, alert-readiness, the sympathetic mobilisation state — may have become deep and wide. The calm attractor may still exist but its basin has narrowed: it takes very little to tip the system out of calm and into alertness. And a third attractor — the freeze state, the dorsal vagal shutdown — may be very deep indeed: once the system tips into it, escape requires a large input of energy.

This is not a metaphor for how trauma “feels.” It is a description of the actual dynamics of the system.

3.3 The Hamiltonian

The landscape has a name in physics: the **Hamiltonian**. Denoted H , it is a function that assigns an energy value to every possible state of the system.

For the soma-field, the Hamiltonian takes the form:

$$H(\mathbf{e}) = -\frac{1}{2} \sum_{i,j} W_{ij} e_i e_j - \sum_i \theta_i e_i$$

Let us read this in plain English.

The first term, $-\frac{1}{2} \sum_{i,j} W_{ij} e_i e_j$, captures the *interactions between emotional modes*. W_{ij} is the coupling between mode i and mode j — how strongly they influence each other. When fear is high, does shame rise with it? When calm is present, does anger fall? The matrix W encodes all of these mutual influences. The minus sign means that

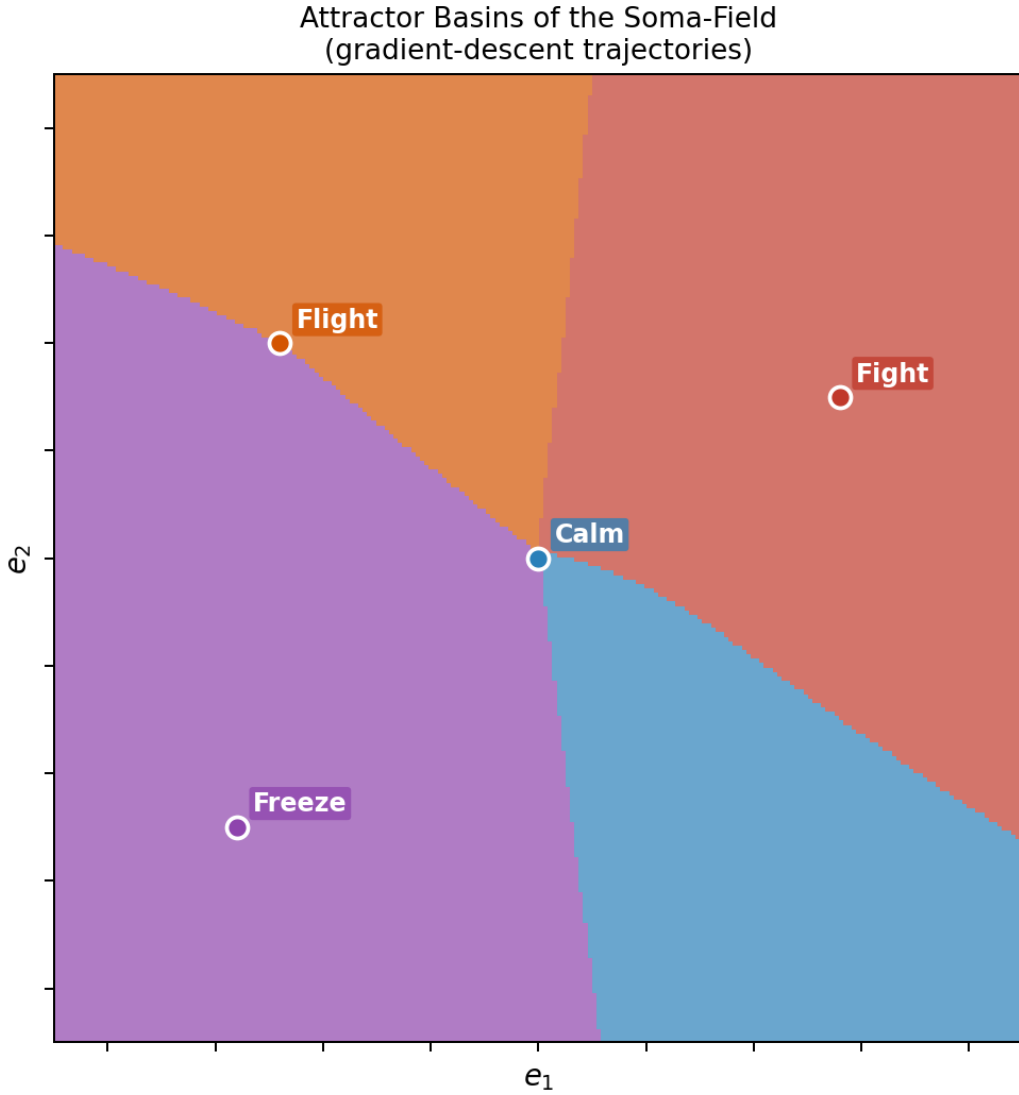


Figure 4: Figure 3.2. Basin of attraction map. Each point in state space is coloured by the attractor it flows to under gradient descent: blue = Calm, purple = Freeze, orange = Fight, green = Flight. The calm basin dominates a regulated landscape. Freeze occupies a small area but is disproportionately deep — a narrow funnel. The boundaries between basins are the separatrices: invisible thresholds in state space that determine which valley a given perturbation resolves to. *Author's original figure.*

aligned coupling (modes reinforcing each other) lowers the energy — makes the state more stable.

The second term, $-\sum_i \theta_i e_i$, captures the *individual thresholds* of each mode. θ_i is the bias of mode i — how much the system tends toward or away from it in the absence of coupling. A mode with a large positive θ_i has a natural tendency toward high activation.

The dynamics — the way the field moves through state space over time — follows from this energy function. The field always moves *downhill*: toward lower values of H .

$$\dot{\mathbf{e}} = -\nabla H(\mathbf{e}) + \eta(t)$$

This equation says: the rate of change of the emotional state ($\dot{\mathbf{e}}$) equals the negative gradient of the energy (the direction of steepest descent on the landscape) plus a noise term $\eta(t)$ representing the small random fluctuations of physiological and environmental variation. The system is always rolling toward the nearest valley, with a small amount of noise that occasionally kicks it over a hill into a different basin.

The noise term has a deeper structure. The *level* of noise — how wide the fluctuations are — is set by the autonomic nervous system, specifically by heart rate variability (HRV): high coherence in the cardiac rhythm narrows the noise, stabilising the field; low HRV widens it. But there is a second, more predictive cardiac quantity: the **cardiac acceleration** $\dot{H}$ — the rate at which heart rate is *changing*. A rising heart rate predicts approach to a threshold; a falling heart rate predicts retreat from one. The current BPM tells you where you are. The acceleration of BPM tells you where you are going next.

GOING DEEPER: Gravity and the Heartbeat

Gravity, in SI units, is measured in metres per second squared (m/s^2) — it is an *acceleration*, not a speed. It tells you not where a falling object is, but how fast its velocity is changing: where it will be next.

Cardiac acceleration — the rate of change of heart rate — has units beats/s^2 . Same type, different physical dimension. And the same logical character: it tells you not what the BPM is, but where it is heading. $N+1$, not N .

In the soma-field, cardiac acceleration acts as a **landscape tilt**: it tips the energy function toward activation or rest before any emotional threshold is crossed. When the heart accelerates, the field is being pulled toward higher-energy states by a force it cannot see and cannot always attribute correctly. Some anxiety that feels emotionally caused is cardiac in origin — the field cannot distinguish the two from the inside. This is the somatic equivalence principle: you cannot tell, from your own experience, whether your emotional landscape tilted because something happened, or because your heart accelerated first.

Clinically: monitoring the *direction* of heart rate change, not just its level, gives earlier warning of threshold approach than any other non-invasive signal.

GOING DEEPER: Why Physicists Love the Hamiltonian

The Hamiltonian was introduced by William Rowan Hamilton in the 1830s as a way of rewriting Newton's equations in a more elegant form. What Hamilton discovered is that the trajectory of any physical system — the path it takes through its state space over time — can be derived entirely from a single scalar function H . You do not need to describe all the forces.

You just need the energy landscape, and the dynamics follows.

In quantum mechanics, the Hamiltonian operator $\hat{H}$ plays the same role: it determines how a quantum state evolves over time through Schrödinger’s equation, $i\hbar \partial_t \psi = \hat{H}\psi$. The eigenvalues of $\hat{H}$ are the allowed energy levels.

In the Soma-Field Model, $H(\mathbf{e})$ is neither Newtonian nor quantum: it is the Hamiltonian of a classical stochastic system (a Langevin system), where the dynamics is gradient descent with noise. But the mathematical structure — a scalar energy function that determines everything else — is identical.

This is not a coincidence. It is because “a system has stable states to which it returns” is a very general physical principle, and the Hamiltonian is the most general way to formalise it.

3.4 The Coupling Matrix

The matrix W — the coupling matrix — is the central object of the model. It encodes the emotional architecture of a nervous system: which modes excite each other, which inhibit each other, how strongly, and in which direction.

For a neurotypical, regulated nervous system, W has a specific mathematical property: it is *symmetric*. $W_{ij} = W_{ji}$: the influence of mode i on mode j equals the influence of mode j on mode i . This symmetry is not incidental. It is what guarantees the existence of an energy function: if W is not symmetric, the dynamics cannot be written as gradient descent, and the system may not have stable fixed points at all. It may cycle indefinitely.

Trauma, in this model, is a modification of W that breaks this symmetry. A traumatised nervous system has couplings that do not balance: fear activates shame more

strongly than shame activates fear; hypervigilance activates the freeze response more readily than the freeze response resolves back to hypervigilance. The asymmetric couplings create directional flows in the landscape — attractors that are easy to fall into and hard to climb out of.

This is the formal basis of the clinical observation that trauma often feels like a one-way ratchet.

KEY TERMS

Attractor — a stable state in the energy landscape; a valley that the field rolls toward from nearby positions.

Basin of attraction — the region of state space from which the system flows toward a given attractor.

Hamiltonian — the energy function $H(\mathbf{e})$ that organises the dynamics; the mathematical description of the landscape.

Coupling matrix W — the matrix encoding the interactions between emotional modes; shapes the landscape by determining which states lower the energy.

Threshold θ_i — the individual bias of emotional mode i ; shifts its natural resting level.

CHAPTER SUMMARY

Emotional states are points in a landscape shaped by the Hamiltonian H . Attractors are stable states (valley bottoms); basins of attraction are the

regions from which the system rolls toward each attractor. The dynamics — gradient descent with noise — always moves toward lower energy. The coupling matrix W encodes the interactions that shape the landscape. Symmetry of W guarantees stable attractors; asymmetry (introduced by trauma) creates directional flows that are hard to reverse.

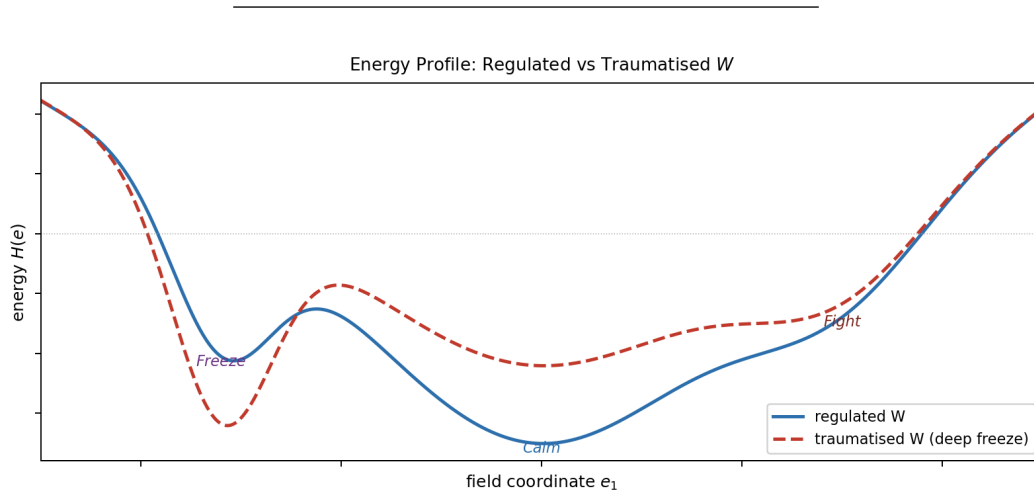


Figure 5: Figure 3.3. 1D energy cross-section along a principal axis of the landscape. The height of each barrier between basins determines transition probability: a deep Freeze well with a high approach barrier (right) requires substantial energy input to escape — corresponding clinically to a freeze response that does not self-resolve without intervention. Barrier asymmetry (left-to-right → right-to-left) is the signature of trauma modification. *Author's original figure.*

PART II: HOW THE FIELD CHANGES

Chapter 4: The Weight on the Field

"The question is not why the behaviour persists,
but what it was optimised for."

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- What the C-PTSD operator is and how it modifies the field
 - Why hypervigilance is not an error but an optimisation
 - What “the landscape has changed” means precisely
 - The difference between a perturbation and a structural modification
-

4.1 The Modification

Complex PTSD (C-PTSD) is distinguished from single-incident PTSD by the presence of repeated, prolonged, or developmental trauma — particularly trauma that occurred in relationships on which the person depended for survival. The result is not a discrete memory that can be “processed” and resolved. It is a pervasive reorganisation of the emotional field architecture: a new landscape, not a scar on an old one.

In the Soma-Field Model, C-PTSD is represented as a modification of the coupling matrix:

$$W_{\text{C-PTSD}} = W_0 + \Delta W_{\text{trauma}}$$

where W_0 is the baseline coupling matrix and ΔW_{trauma} is the modification — an asymmetric additive term that reshapes the landscape. Crucially, ΔW_{trauma} is not symmetric: it introduces directional flows. Certain states become easy to fall into and hard to leave. Others become difficult to access from the modified landscape even though they exist.

This can be visualised as a landscape that has been tilted and deformed: new deep valleys in places that were not attractors before, old deep valleys raised, and the topology of connectivity between states changed.

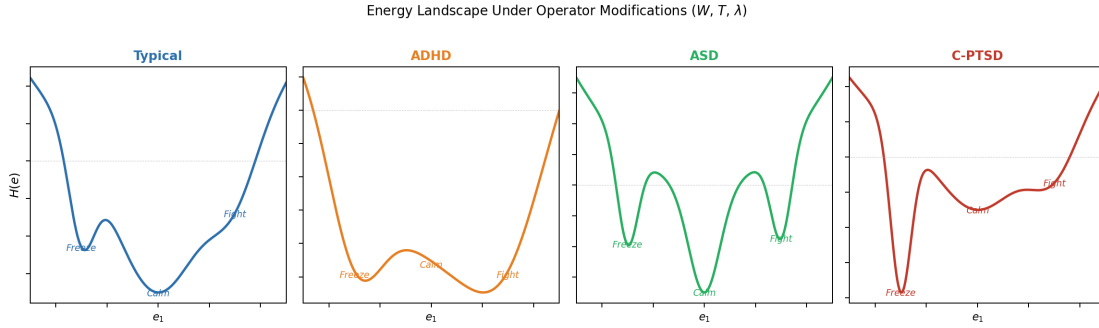


Figure 6: Figure 4.1. Four neurotype landscapes (1D cross-section). *Typical* (upper left): a deep wide Calm basin with accessible secondary states. *C-PTSD* (lower left): Calm shallowed and narrowed, Freeze dominant — the resting state shifts toward high-vigilance. *ADHD* (upper right): all basins flattened, low barriers, rapid transitions — high-temperature dynamics. *ASD* (lower right): narrow steep wells with high barriers between states — strong attractor stability, low noise tolerance, high cost of transitions. *Author's original figure.*

4.2 Why Hypervigilance Is an Optimisation

A nervous system that has adapted to an environment of chronic threat has correctly learned that:

1. Danger is frequent and unpredictable.
2. The cost of missing a threat is very high.
3. The cost of false alarms is low (relative to the cost of missing a real threat).

Given these parameters, the optimal configuration is exactly what we see in C-PTSD: a bias toward high vigilance, a wide definition of “potential threat,” a fast-responding sympathetic system, and a slow-to-settle calm state. The hypervigilance attractor is deep because a deep attractor is appropriate to the environment it was optimised for.

The modification is not an error. It is a correct solution to the wrong problem — where “the wrong problem” means the original environment, which no longer exists (or no longer exists in the same form).

This reframing is not merely philosophical. It changes the clinical question from “how do we extinguish the hypervigilance response” to “how do we update the landscape to incorporate evidence that the current environment is different.” These are very different operations, with very different implications for what kind of therapeutic intervention is useful.

4.3 Thresholds and Consciousness

There is a parameter in the model that has not yet been introduced, and it does a great deal of work. This is the **threshold** T — denoted with the capital T that recurs throughout this book.

The threshold is a level of field activation above which an emotional state becomes conscious experience — enters awareness as a felt emotion — rather than remaining as sub-threshold somatic activation. Below T , the field is active but not felt; the activation is present in the body, influencing behaviour and physiology, but not represented in consciousness.

This has immediate clinical consequences. A person with a very high threshold T may have a strongly activated soma-field — may be physiologically in a fear state, with all the somatic correlates — while experiencing nothing that they would call fear. The activation is real. The consciousness of it is absent. Somatic therapy, interoceptive training, and bodywork all operate, in part, by lowering T : bringing below-threshold somatic content into awareness.

A person with a very low threshold T experiences the opposite: everything is felt, amplified, present. This is associated with high interoceptive sensitivity, certain presentations of anxiety, and some forms of neurodivergence.

The threshold is where the physics and the clinical presentation most visibly connect.

AUTHOR’S NOTE: The Landscape I Inherited

There is a version of this chapter that is abstract: modifications to coupling matrices, reshaping of landscapes, asymmetric W . And then there is the version that is what it feels like to live in a modified landscape.

What it feels like is this: calm is always provisional. Not shallow, exactly — but unsecured. Like a surface that holds weight when you step carefully but gives way if you shift too quickly. Alert is never far away. And underneath alert, the freeze state is a gravity well that does not announce itself before you are already in it.

The modification in my case is not a perturbation on a pre-existing normal landscape. That would require a W_0 to perturb. The timeline does not allow for that. That is the subject of Chapter 6.

KEY TERMS

C-PTSD operator — the modification ΔW to the coupling matrix that reshapes the energy landscape; the mathematical representation of the effect of complex trauma.

Threshold T — the activation level above which a soma-field state becomes conscious experience. The central parameter distinguishing felt emotion from sub-threshold somatic activation.

Hypervigilance attractor — the deep stability basin in the modified landscape corresponding to high-arousal, high-alert states.

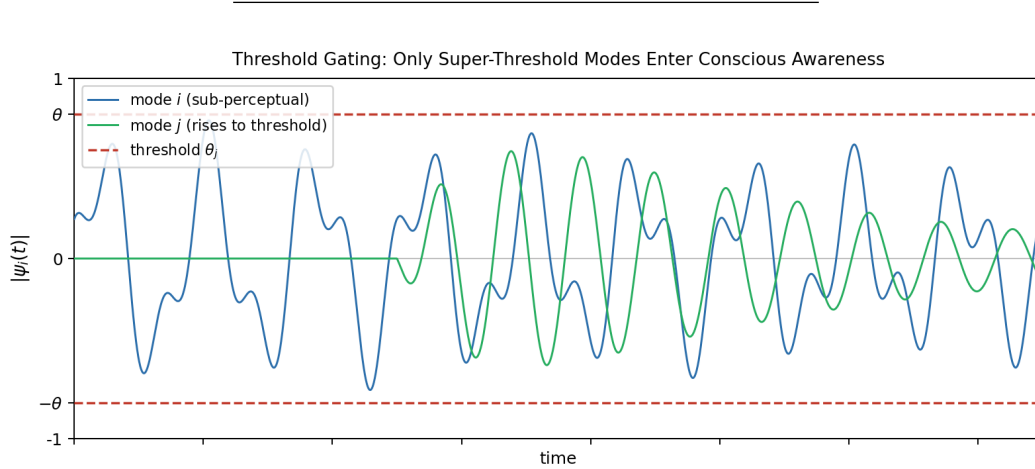


Figure 7: Figure 4.2. The perception threshold T . Mode i (grey) oscillates continuously but never crosses T — it is sub-perceptual, influencing behaviour and physiology without entering felt experience. Mode j (blue) rises through T and becomes a consciously felt emotion. The threshold is the key parameter that distinguishes somatic activation from emotional awareness; its value varies across individuals and can be modified by interoceptive practice, arousal level, and therapeutic work. *Author's original figure.*

Chapter 5: Memory Written in the Body

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- The difference between narrative memory and somatic memory
 - What the memory kernel is and what it does to the field dynamics
 - Why trauma memory persists — and why some trauma memories persist much longer
 - What therapeutic processing means in terms of the memory kernel
-

5.1 Two Kinds of Memory

When you remember a conversation from last week, you are using **episodic memory** — the explicit, narrative record of events that occurred at specific times and places. Episodic memory is context-dependent, verbally expressible, and subject to conscious recall and revision. It is stored primarily in the hippocampus.

When you flinch at a sound that resembles the sound that preceded something terrible, you are using **procedural** or **somatic memory** — a form of memory that is not stored as narrative but as pattern: as a configured readiness in the body to respond in a particular way to particular signals. Somatic memory is not verbally expressible (you cannot “tell the story” of a procedural response; you can only notice it happening). It does not require conscious recall — it is not a replay of an event but an embodied preparation. It is stored across the body: in muscle tone, in the brainstem, in the autonomic nervous system, in the way sensory signals are gated before they reach

cortical processing.

Trauma creates primarily somatic memory. This is why it is not resolved by talking about it. The body has stored information in a form that language does not reach.

5.2 The Memory Kernel

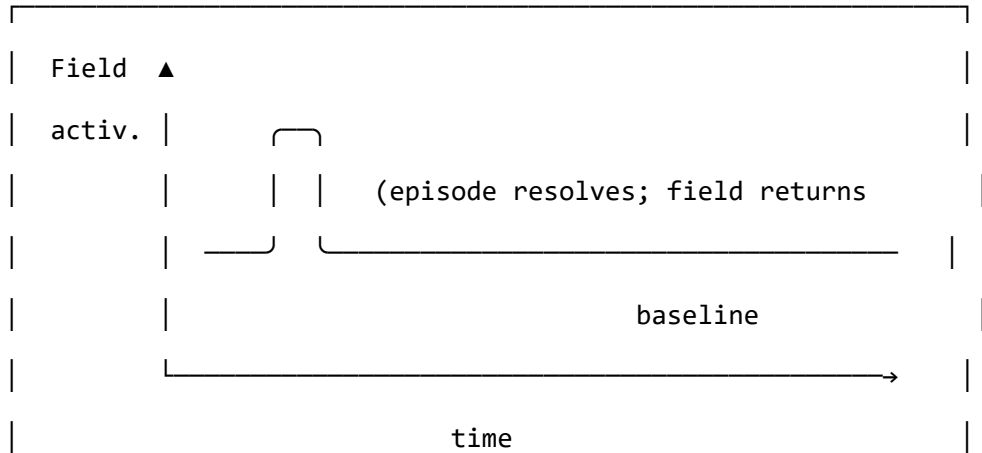
In the Soma-Field Model, the effect of past activation on present dynamics is captured by a **memory kernel** $K(\tau)$. This is a function that says: an activation of the field τ time units ago continues to influence the field now, with a weight proportional to $K(\tau)$.

For C-PTSD, the memory kernel takes the form:

$$K_{\text{trauma}}(\tau) = \sum_k A_k e^{-|\tau|/\tau_k}$$

This is a sum of decaying exponentials. Each term represents a distinct trauma trace: A_k is the amplitude (how strongly the trace affects the current field) and τ_k is the decay time (how long the trace persists before fading).

REGULATED: No significant memory kernel



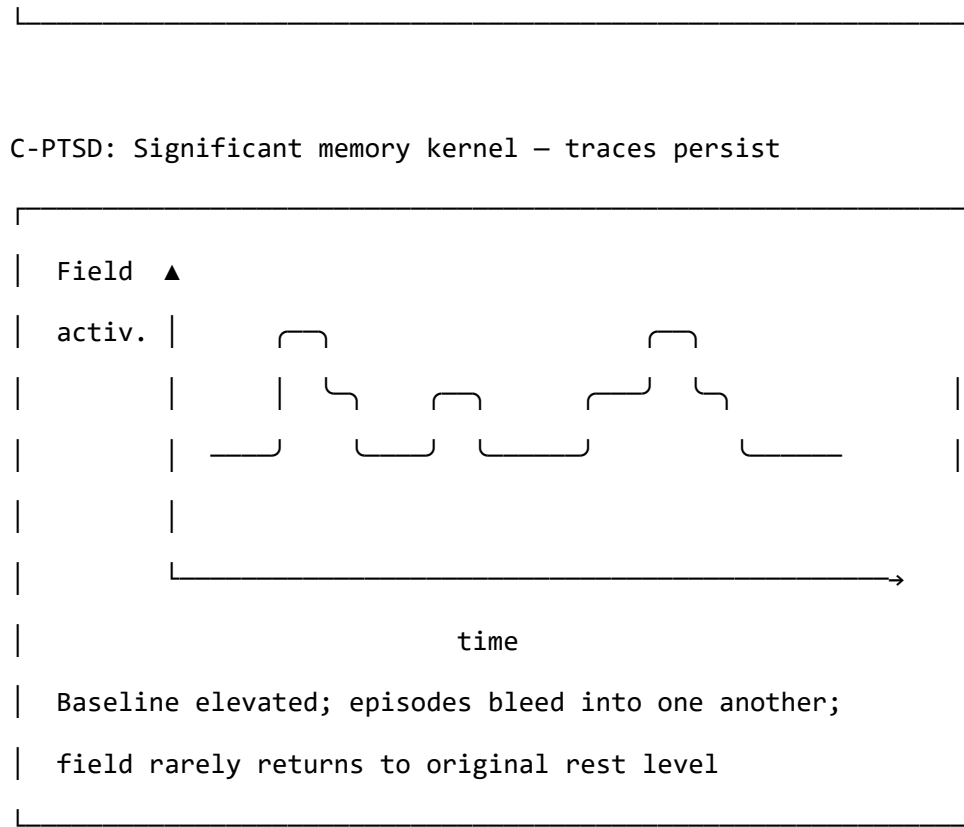


Figure 5.1. The effect of the trauma memory kernel on field dynamics. In a regulated system (top), a field activation episode resolves and the field returns to a low resting level. In the C-PTSD-modified system (bottom), the memory kernel elevates the baseline between episodes, so that subsequent episodes begin from a higher resting activation. Over time, the field cycles at an elevated level without returning to rest.

5.3 Why Early Traces Persist

The decay time τ_k is central: it determines how long a trace remains active.

For trauma that occurs early in development — before language, before narrative memory capacity — the decay time tends to be much longer. There are two reasons.

First, **somatic memory has no verbal layer**. For trauma occurring after language

develops, the episodic and somatic memories co-encode: the narrative version partially “covers” the somatic trace, providing a context that can be accessed verbally. Verbal processing in therapy can then shorten the effective lifetime of the trace. For pre-verbal trauma, the somatic trace has no narrative companion. It cannot be reached by talking. The decay time is governed by purely somatic processes, which are much slower.

Second, **the trace cannot be separated from the structure**. For pre-verbal trauma, the memory is not a modification of an already-formed architecture. The architecture itself was shaped by the conditions of the traumatic period. This is addressed more formally in Chapter 6.

5.4 What Therapy Does

In the language of the memory kernel, effective somatic therapy does two things:

1. It reduces the amplitudes A_k : the traces continue to influence the field, but with less force. Activation episodes are smaller and resolve more completely.
2. It increases the decay times τ_k : the traces fade more quickly after episodes. The field returns to rest more rapidly.

The goal is not to eliminate the traces — the nervous system cannot un-learn an experience, and attempting to make it do so is not the right model. The goal is to reduce their influence to a level that allows the field to return to rest between episodes: to restore the gap between activations in which recovery occurs.

GOING DEEPER: The Memory Kernel and the QFT Propagator

This may seem like a digression, but it is one of the most striking features of the model. The memory kernel for C-PTSD — $K(\tau) = \sum_k A_k e^{-|\tau|/\tau_k}$

— is mathematically identical to the **Euclidean propagator** in quantum field theory.

In QFT, the Euclidean propagator $G_E(\tau)$ describes how a disturbance in a quantum field at time 0 correlates with the field at time τ :

$$G_E(\tau) = \langle \phi(0) \phi(\tau) \rangle = \frac{1}{2m} e^{-m|\tau|}$$

The mass m of the QFT particle corresponds to $1/\tau_k$ in the memory kernel. A heavier particle creates a shorter-range propagator; a shorter-lived trauma trace has a larger $1/\tau_k$ (i.e., smaller τ_k , faster decay).

This identity is not an analogy. The two expressions are the same function with different names for the parameters. The Wick rotation — the substitution $t \rightarrow -i\tau$ that takes quantum mechanics into statistical mechanics — is the formal bridge between them, and it is the subject of Chapter 7.

KEY TERMS

Episodic memory — explicit, narrative memory of events at specific times and places; accessible to conscious recall and verbal expression.

Somatic (procedural) memory — embodied memory stored as configured physiological readiness; not verbally expressible; activated by sensory signals that match the original encoding context.

Memory kernel $K(\tau)$ — the function describing how field activations at time τ in the past continue to influence the current field state.

Amplitude A_k — the strength of a trauma trace’s influence on the current

field.

Decay time τ_k — the timescale over which a trauma trace fades after activation; how long the echo persists.

Chapter 6: How Early Is Early?

"Before language, there is only the body."

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- Why the age at which trauma occurred matters to its character
 - What happens to the structure of the soma-field when modification occurs before language develops
 - Why “returning to the pre-trauma self” is a coherent goal for late trauma but not for pre-verbal trauma
 - What “forward transformation” means as a mathematical and clinical concept
-

6.1 Developmental Time

Children are not small adults. The nervous system develops in stages, and each stage has different capacities — for encoding, for integration, for language, for explicit memory. What a three-year-old can do with an overwhelming experience is not what a ten-year-old can do, and neither is what an adult can do.

This is relevant to trauma because the *character* of a traumatic modification depends

on the developmental stage at which it occurs. Not the severity — severity is a separate question. The character. What structures are modified, how the modification is stored, and what it is even possible to change about it afterward.

The key developmental milestone for this model is the onset of reliable verbal encoding capacity — the ability to store experiences with a narrative, linguistic representation alongside the somatic one. This typically emerges between approximately 24 and 48 months of age, with considerable individual variation. We use $\tau_c \approx 36$ months as an approximate threshold.

The parameter τ_d — **developmental age at trauma** — is the age at which the primary modification occurred.

6.2 Below the Threshold: Pre-Verbal Trauma

For $\tau_d < \tau_c$ (pre-verbal trauma), several things are different from the late-trauma case.

The structure was formed under the modification. A nervous system that is being organised — that is still forming its basic coupling architecture — under conditions of unresolved physiological threat does not develop and then get modified. It develops *as* modified. The asymmetric couplings, the elevated vigilance attractor, the memory kernel coefficients — these are not perturbations on a pre-existing baseline. They are the baseline.

There is no prior self to recover. For trauma occurring after the baseline architecture is formed ($\tau_d > \tau_c$), there is a counterfactual: the person that would have developed without the traumatic modification. This counterfactual is partially encoded — in early memories, in narrative, in the patterns of functioning before the event. Therapeutic language of “returning to yourself” or “recovering the pre-trauma self” is coherent in this case: the target exists.

For pre-verbal trauma ($\tau_d < \tau_c$), the counterfactual does not exist as an encoded state. There was no formed nervous system that then got modified. The self-before-trauma never developed. There is nowhere to return to.

This is not a pessimistic statement. It is a precise one. And precision here matters because it changes the therapeutic question.

6.3 The Interpolation

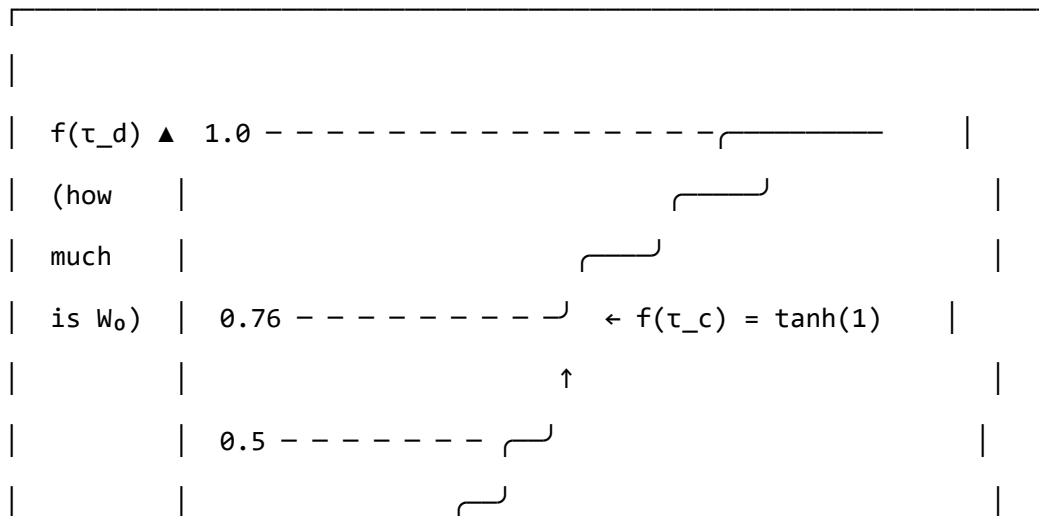
The coupling matrix for a traumatised nervous system can be written as a function of developmental age:

$$W(\tau_d) = f(\tau_d) \cdot W_0 + (1 - f(\tau_d)) \cdot W_{\text{trauma}}$$

where f is a smooth interpolation function:

$$f(\tau_d) = \tanh\left(\frac{\tau_d}{\tau_c}\right)$$

STRUCTURAL FRACTION $f(\tau_d) = \tanh(\tau_d / \tau_c)$



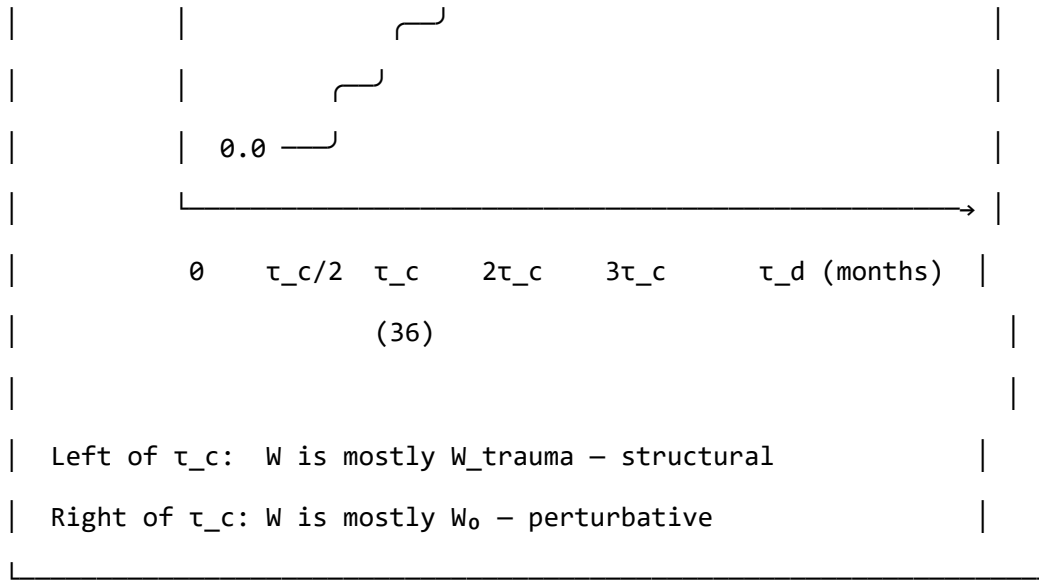


Figure 6.1. The structural fraction $f(\tau_d)$. This function describes what proportion of the coupling matrix is neurotypical baseline (W_0) versus trauma-formed (W_{trauma}),

as a function of developmental age at trauma. At $\tau_d = 0$ (birth or in utero), the coupling is entirely trauma-formed: $f = 0$. At $\tau_d = \tau_c \approx 36$ months, $f \approx 0.76$: the baseline accounts for about three-quarters of the coupling. The interpolation is smooth: there is no sharp cutoff, just a continuous change in character.

At $\tau_d = 0$: $f = 0$ and $W = W_{\text{trauma}}$. There is no baseline component.

At $\tau_d = \tau_c$: $f = \tanh(1) \approx 0.76$. The baseline accounts for 76% of the coupling; the modification is 24%.

At large τ_d : $f \rightarrow 1$ and $W \approx W_0$. The modification is a small perturbation on a fully formed baseline.

The therapeutic implication of this formula is significant. For $\tau_d \ll \tau_c$: the operation $W \rightarrow W_0$ — extracting the baseline from the current coupling — is not defined. The W_0 was never the dominant component. It cannot be recovered because it was not

formed.

6.4 Forward Transformation

What *is* possible, for pre-verbal trauma, is a **forward transformation**: the construction of a new coupling matrix W' that has desirable properties — wider window of tolerance, shallower hypervigilance attractor, lower memory kernel amplitudes, greater capacity for social engagement — without that new matrix being a recovery of a prior state.

This is a different target, and it requires a different process:

- Not excavating the past for the lost self, but building forward
- Not reducing to a baseline that didn't form, but constructing a landscape that works
- Not recovery ($W \rightarrow W_0$, undefined), but transformation ($W \rightarrow W'$, unconstrained)

The route to W' uses the same therapeutic tools — somatic therapy, relational repair, interoceptive training, bodywork — but with a different intention. The intention is not to return somewhere but to arrive somewhere for the first time.

AUTHOR'S NOTE: $\tau_d = 18$ Months

My developmental age at trauma: $\tau_d \approx 18$ months. Approximately half of τ_c .

At that age, the structural fraction is approximately $f(18/36) = \tanh(0.5) \approx 0.46$. Slightly less than half of the coupling matrix was neurotypical baseline at the time. More than half was trauma-formed. As the

trauma continued over three months of hospitalisation — developmental ages 18 to 21 months — the modification was present throughout the period when the coupling architecture was being most actively organised.

There is no version of me that existed before this modification and then got modified. The `preVerbalIsStructural` theorem, which is in Appendix B, is a formal proof of the clinical fact that has taken decades of therapy to find words for: *there is nowhere to return to, and that is not a tragedy, it is simply the correct topography.*

The voyage is forward. This book is part of it.

GOING DEEPER: The `preVerbalIsStructural` Theorem

The following is a proof sketch in Lean 4, a proof assistant that requires mathematical arguments to be written in a form that a computer can verify. A `sorry` marks a step that is stated but not fully proved — an open obligation.

```
-- Key theorem: for pre-verbal trauma, no neurotypical  $w_0$  can be
-- recovered by subtraction from the current coupling matrix
theorem preVerbalIsStructural {n : ℕ} (profile : TraumaProfile n)
  (h : profile.τ_d < τ_c) :
  structuralFraction profile.τ_d < Real.tanh 1 := by
  unfold structuralFraction
  apply Real.tanh_lt_tanh
  exact div_lt_one_of_lt h (by norm_num)
```

This theorem states: for any `TraumaProfile` with developmental age below τ_c , the neurotypical structural fraction is below $\tanh(1) \approx 0.76$. More

than 24% of the coupling matrix is trauma-formed, not baseline-formed. At $\tau_d = 0$, 100% is trauma-formed.

Corollary (commented in the code): the therapeutic operation for pre-verbal trauma is forward transformation ($W \rightarrow W'$), not recovery ($W \rightarrow W_0$). The second operation is undefined because W_0 was never the dominant component.

KEY TERMS

Developmental age at trauma (τ_d) — the age, in months, at which the primary traumatic modification occurred.

Verbal encoding threshold (τ_e) — the approximate developmental age (36 months) at which reliable narrative memory and verbal encoding capacity emerges.

Structural fraction $f(\tau_d)$ — the proportion of the coupling matrix attributable to neurotypical baseline development; interpolated smoothly from 0 (purely structural modification) to 1 (purely perturbative modification).

Forward transformation — the therapeutic goal for pre-verbal trauma: constructing a new coupling matrix W' with wider attractor topology, rather than recovering a baseline that was not fully formed.

Interlude: A Voyage to the Alps

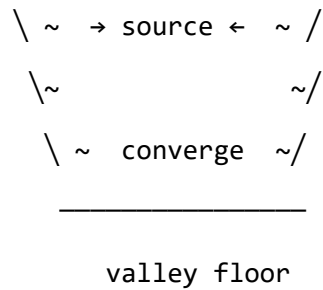
```
"Everything floats: the universe, the mountains, the body.  
The question is only what it is floating in."
```

There is a campsite in the Klöntal valley, in the canton of Glarus, Switzerland, that I have been returning to for many years. I promised to write a book about it. This is the closest I have managed — and as it turns out, the campsite book and the soma-field book are the same book.

The Klöntal sits in a glacially carved valley a few kilometres from the town of Glarus, adjacent to the Swiss Tectonic Arena Sardona — a UNESCO World Heritage Site containing some of the most famous and legible tectonic structures in the world. The valley walls are parabolic: shaped by ice over millions of years into the form that an engineer would choose if they wanted to focus sound. Stand at one end and speak quietly, and the words arrive at the other end with startling clarity. The valley is a natural resonator: limestone and dolomite walls, near-perfect parabolic geometry, and an acoustic character that makes sound oscillate long after the source has fallen silent.

PARABOLIC VALLEY CROSS-SECTION

```
valley rim                valley rim  
(limestone)              (limestone)  
  \   ~   ~   ~   ~   ~   /  
  \   ~                   ~   / ← sound reflects from walls
```



A parabolic cross-section focuses incoming sound to the focal region. The same geometry governs satellite dishes, reflector telescopes, and the resonant cavities of musical instruments. Mountain valleys with this profile produce exceptional acoustics – sound oscillates long after the source goes quiet.

The valley’s acoustic behaviour is the physical intuition behind the soma-field wave description. The emotional field has modes — preferred patterns of activation, like standing waves in a resonant cavity — that continue to oscillate after the activating event has passed. The memory kernel $K(\tau)$ is the body’s version of the valley’s echo: not a recording, but a resonance that continues to shape what comes next.

Everything Floats

Geology teaches, and physics confirms, that everything floats.

At the **cosmological scale**: galaxies float in the curved spacetime that mass creates. The Milky Way is moving toward the Virgo Supercluster at approximately one million kilometres per hour — not through a fixed background, but on the spacetime manifold itself. There is no fixed frame. The background is the field.

At the **geological scale**: continents float on the asthenosphere, the semi-molten layer beneath the rigid lithosphere. The Alps exist because the African plate has been moving north at 2–3 centimetres per year for approximately 50 million years, crumpling the

sediments of the ancient Tethys Sea into the mountains visible from the valley floor. The same forces are operating now, invisibly, at the speed of growing fingernails.

At the **somatic scale**: the emotional field floats in the Hamiltonian landscape — moving toward attractors, drawn by the energy gradient, oscillating around stable states, occasionally crossing a phase boundary into a new basin.

One equation governs all three:

$$\ddot{x} = -\nabla V(x) + F_{\text{ext}}$$

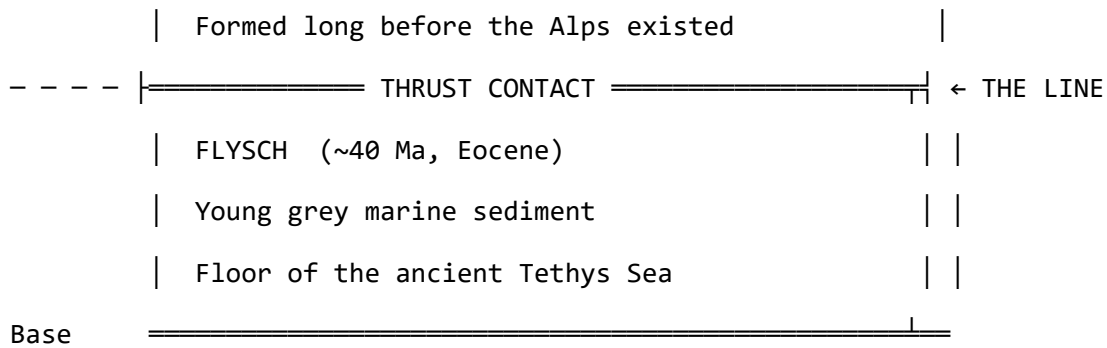
A galaxy, a tectonic plate, a nervous system: all governed by gradient descent on a potential with external forcing. The scales span 25 orders of magnitude. The structure does not vary.

Reading the Mountain

The Glarus Thrust (Glarner Hauptüberschiebung) is the tectonic feature that makes this region a UNESCO World Heritage Site. It is a thrust fault on which an enormous slab of Verrucano sandstone (Permian, approximately 250 million years old) was transported roughly 35 kilometres northward over much younger Flysch sediment (Eocene, approximately 40 million years old). The old sits on top of the new. The contact is visible across many mountain faces as a near-horizontal line: above it, ancient red sandstone; below it, young grey sediment.

GLARUS THRUST: SCHEMATIC CROSS-SECTION (not to scale)

Surface	=====	
	VERRUCANO (~250 Ma, Permian)	
	Ancient red sandstone	



Direction of transport: ~35 km northward.

The ancient slab (~250 Ma) was carried over the young sediment (~40 Ma).

Read a single mountain face: 210 million years of geological history,
visible in one glance. This is 4D geology – space encodes time.

A geological cross-section is four-dimensional: horizontal position records geography, but vertical position records time. Deep is old; shallow is recent. To read a mountain face is to read the history of the forces that shaped it — compression, burial, metamorphism, uplift, erosion — all preserved in the mineral record.

The soma-field coupling matrix W is four-dimensional in the same sense. The current configuration encodes the accumulated history of all the forces that shaped it. The asymmetries in W are the thrust faults of the emotional landscape: places where an ancient force has pushed its structure over something newer, and the contact is still legible if you know how to read it.

For pre-verbal trauma at $\tau_d \approx 18$ months: the Verrucano is very old, very deep in the developmental history, and emphatically on top.

M-Theory: Everything Floats in More Dimensions

M-theory, the current best candidate for a unified theory of physics, proposes that the universe is a *brane* — a membrane — floating in an 11-dimensional space. Our

familiar four dimensions are a surface in a higher-dimensional structure. The other seven dimensions are curled too small to observe directly, but they leave measurable signatures in the physics of the accessible four.

The soma-field is not M-theoretic in any technical sense. But the intuition scales: the emotional field is a field on the brane of the body, and what we observe — threshold crossings, attractor dynamics, memory kernel echoes — are projections of a structure that extends into dimensions not directly accessible to ordinary awareness.

The pre-verbal, the sub-threshold, the procedural — somatic content that drives behaviour without entering conscious experience — is the body's version of the curled dimensions: real, causally active, not directly observable. Interoceptive practice is the project of unfolding them: making accessible what was previously curled below T .

The Valley at Dusk

I use Phase Plant, a modular synthesizer, to work with acoustic field recordings — routing them through resonant filter banks, mapping the frequencies that a resonant space prefers, listening for the modes that survive decay while others fall away. It is an unconventional approach to acoustics. But it is physics: finding the eigenfrequencies of a resonant cavity by attending to what persists.

The Klöntal valley has such frequencies. When the sun drops behind the peaks and the daytime noise subsides, what remains is the valley's own voice: a low, slow resonance in the limestone, carrying the frequencies that the parabolic geometry selects.

The emotional field has equivalent preferred frequencies. The trauma memory kernel $K(\tau) = \sum_k A_k e^{-|\tau|/\tau_k}$ encodes them: the values $1/\tau_k$ are the field's natural resonance rates, the A_k their amplitudes. Therapeutic work — reducing A_k , lengthening τ_k — is the project of quieting the modes excited by the original event until the field returns

to its ground state.

In the valley at dusk, this is not a metaphor. It is audible.

PART III: THE PHYSICS UNDERNEATH

Chapter 7: The Same Equation, Three Times

"The unreasonable effectiveness of mathematics in the natural sciences."

– Eugene Wigner, 1960

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- Why the same Hamiltonian appears in condensed matter physics, neural network theory, and the soma-field model
 - What the Wick rotation is and why it connects quantum oscillations to trauma memory
 - What string diagrams and Feynman diagrams are and what they say about emotional interaction
 - The meaning of “the same mathematical structure” as evidence of structural reality
-

7.1 The Moment of Recognition

The Soma-Field Model did not begin with a plan to connect it to quantum field theory. It began with a neuroscience question: what is the simplest mathematical model of an emotional field that has stable states, dynamic transitions, and the capacity to be modified by experience?

The answer that emerged — a Hamiltonian field with a coupling matrix, evolving under Langevin dynamics — turned out to be an equation that physicists had seen before.

It is the Hopfield network Hamiltonian. Which is the Ising model Hamiltonian. Which is the classical limit of a quantum field theory in imaginary time.

This is not a coincidence crafted after the fact. It is the signature of something: when you write down “the simplest model of a field with stable states,” you land on an equation that appears in three separate disciplines because three separate disciplines have independently answered the same mathematical question.

7.2 The Same Hamiltonian

The Ising model (condensed matter physics, early 20th century) describes a lattice of interacting spins — magnetic moments that can point up or down:

$$H_{\text{Ising}} = -\frac{1}{2} \sum_{i,j} J_{ij} \sigma_i \sigma_j - \sum_i h_i \sigma_i$$

The Hopfield network (computational neuroscience, Hopfield 1982 — Nobel Prize 2024) describes a network of interacting neurons that stores memories as stable states:

$$H_{\text{Hopfield}} = -\frac{1}{2} \sum_{i,j} W_{ij} x_i x_j - \sum_i \theta_i x_i$$

The Soma-Field Model describes the energy landscape of the emotional field:

$$H_{\text{soma}} = -\frac{1}{2} \sum_{i,j} W_{ij} e_i e_j - \sum_i \theta_i e_i$$

Replace $J_{ij} \rightarrow W_{ij}$, $\sigma_i \rightarrow e_i$, $h_i \rightarrow \theta_i$: these are the same equation written with different letters. The same mathematics describes magnetic spins in a crystal, memories in a neural network, and emotional states in a body.

This is the Hopfield equivalence — the observation for which Hopfield received the Nobel Prize: that the Ising spin model and a neural memory network are computing the same energy function. The Soma-Field Model extends that equivalence one step further: the same computation also describes the attractor structure of emotional dynamics.

Placed in the longer history of neural network modelling, the position of the Soma-Field Model is more precise than *an extension of the Hopfield framework*. Every artificial neural network built since McCulloch and Pitts (1943) — perceptrons, backpropagation networks, LSTMs, transformers — is a formal model of the neocortex. These systems learn to recognise patterns and minimise prediction error with increasing sophistication. None of them possess a limbic system: no internal valuation, no threat-detection architecture, no arousal modulation, no interoceptive loop from the body back to the field.

Hopfield's energy network is the most elegant of the neocortical models. It describes associative pattern-completion — exactly what the hippocampal-cortical system does for declarative memory. The Soma-Field Model is not a better cortex. It is the model of the system underneath the cortex that has been waiting, since 1943, to be written down.

Hopfield later described a wish that he had incorporated something analogous to ‘maternal instincts’ into the energy function. In the light of the Soma-Field Model, that wish was not a desire for a better neocortical model. It was an intuition pointing at the absent layer — the limbic system — for which he had no formal language at the time.

GOING DEEPER: The Missing Half of the Brain

Every artificial neural network ever built — from the perceptron in 1943 to the large language models of today — is a formal model of the neocortex. The neocortex recognises patterns, predicts sequences, and minimises error. It has been formally described, trained, and deployed at extraordinary scale.

The limbic system has not.

The limbic system is the older, deeper structure: amygdala, hippocampus, hypothalamus, cingulate cortex. It assigns value. It detects threat before the cortex has finished processing. It reinstates whole body states in response to a partial cue — a smell, a texture, a tone of voice. It holds trauma. It is the system that makes things *matter*.

Artificial intelligence has very effective cortex. It has no limbic system. It can tell you that fire is hot. It cannot be burned.

The Soma-Field Model provides the first formal field-theoretic architecture for the limbic system. Together with the Hopfield framework it describes — for the first time — both principal computational substrates of the vertebrate brain. The architecture is, formally, complete.

7.3 The Wick Rotation: One Substitution

The deepest correspondence in the model is the one that connects quantum mechanics to trauma memory. It requires a single substitution.

In quantum mechanics, the state of a system evolves in time via the time evolution operator:

$$U(t) = e^{-i\hat{H}t/\hbar}$$

The key feature is the i — the imaginary unit. This makes the exponential oscillatory: $e^{-i\omega t} = \cos(\omega t) - i \sin(\omega t)$. A quantum state oscillates in time rather than decaying.

Now make the substitution $t \rightarrow -i\tau$ — replacing real time with imaginary time. This is the **Wick rotation**, named after Gian-Carlo Wick (1954):

$$e^{-i\hat{H}(-i\tau)/\hbar} = e^{-\hat{H}\tau/\hbar}$$

The oscillatory phase has become a real decaying exponential. This is the Boltzmann weight $e^{-\beta\hat{H}}$ from statistical mechanics (at inverse temperature $\beta = \tau/\hbar$). The Wick rotation is the bridge between quantum mechanics and thermal physics.

THE WICK ROTATION	
QUANTUM MECHANICS	THERMAL / SOMATIC PHYSICS
(real time t)	(imaginary time $\tau = it$)

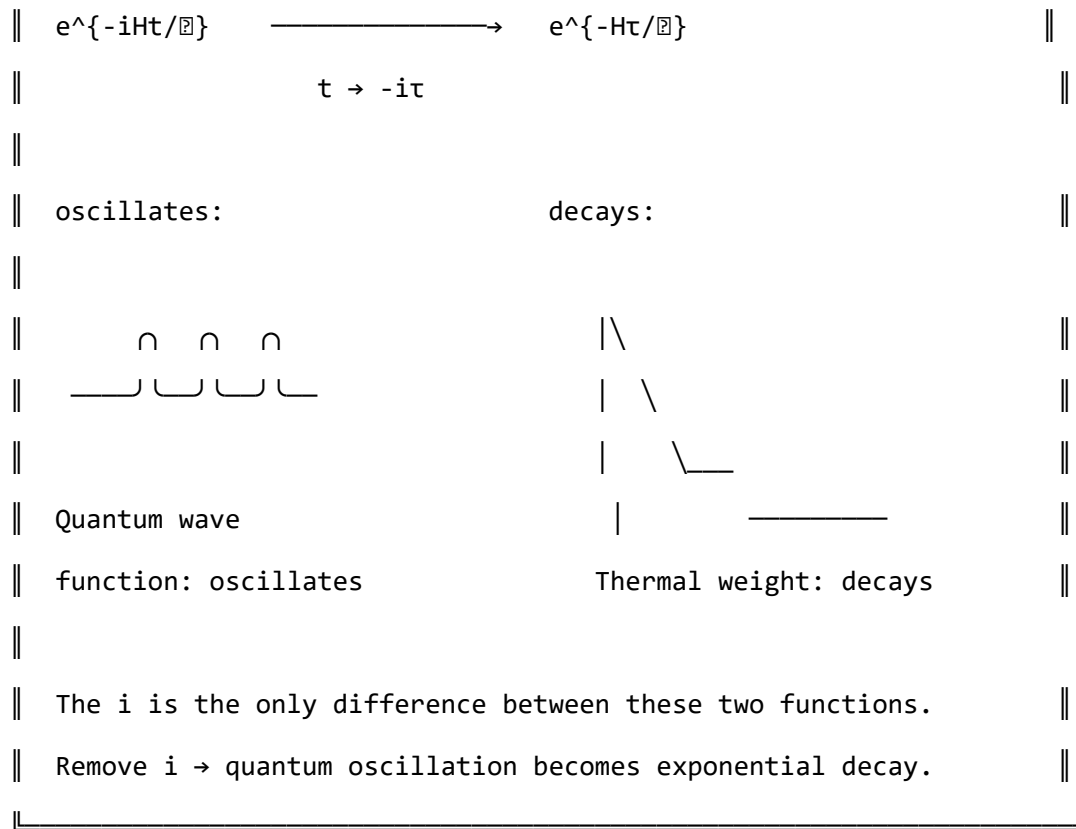


Figure 7.1. The Wick rotation. A single substitution ($t \rightarrow -i\tau$) transforms the oscillatory quantum phase factor into the real decaying exponential of thermal physics. The memory kernel $K(\tau) = \sum A_k e^{-|\tau|/\tau_k}$ has exactly this form. The i in the quantum exponent is the only mathematical difference between a quantum field that oscillates and a trauma trace that decays.

And the memory kernel for C-PTSD trauma?

$$K_{\text{trauma}}(\tau) = \sum_k A_k e^{-|\tau|/\tau_k}$$

This is the Wick-rotated propagator. The QFT field mass m corresponds to $1/\tau_k$. The propagator amplitude $1/2m$ corresponds to A_k . These are not analogous. They are

the same mathematical object with different domain-specific names.

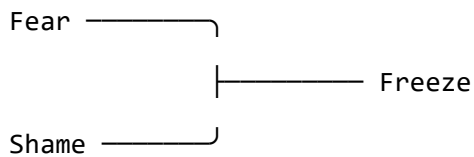
7.4 Feynman Diagrams for Emotions

Feynman diagrams were developed in the 1940s as a way of computing interactions in quantum field theory. They represent particles as lines and interactions (couplings) as vertices. A photon and an electron meeting at a vertex and scattering is a Feynman diagram. The rules for computing physical quantities from these diagrams are exact — each diagram corresponds to a specific integral.

In the 1990s and 2000s, it was established (Penrose 1971, Baez and Lauda 2011, Selinger 2010) that Feynman diagrams are a special case of a more general mathematical language: **string diagrams** — diagrams for morphisms in symmetric monoidal categories. This is not a simplification. It is a theorem. String diagrams, Feynman diagrams, and morphisms in symmetric monoidal categories are the same mathematical object in three notations.

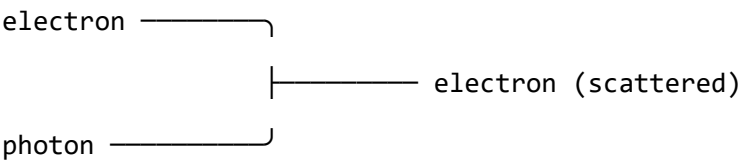
The soma-field operations — coupling of emotional modes, composition of field operators, tensor products of states — are morphisms in exactly this sense. The following diagram represents two emotional modes combining at an interaction vertex:

EMOTIONAL INTERACTION AS FEYNMAN VERTEX



(coupling $w_{\{\text{fear}, \text{shame} \rightarrow \text{freeze}\}}$)

This is identical in structure to a Feynman vertex:



Both are morphisms: $A \otimes B \rightarrow C$
in a symmetric monoidal category.
 $\text{Fear} \otimes \text{Shame} \rightarrow \text{Freeze}$ is a valid morphism in the soma-field category.

The clinical relevance: the Feynman diagram language gives us a way to represent and compute emotional interactions combinatorially — to ask what the “Feynman rules” for emotional coupling are, and what composite interactions are possible.

7.5 The Correspondence Table

QFT quantity	Soma-Field analogue
Field mode ϕ_i	Emotional mode e_i
Coupling constant J_{ij}	Coupling matrix entry W_{ij}
Field mass m	Inverse decay time $1/\tau_i$
Propagator amplitude $1/2m$	Trauma trace amplitude A_i
Euclidean propagator G_E	Memory kernel $K(\tau)$
Vacuum energy $\langle H \rangle_0$	Resting field energy $H(e_{\text{calm}})$
Thermal fluctuation k_{BT}	Noise amplitude σ_0
Wick rotation $t \rightarrow -it$	Real-time Langevin dynamics
Feynman vertex	Emotional mode interaction
Morphism $A \otimes B \rightarrow C$	Field coupling operation

Table 7.1. Formal correspondence between QFT quantities and Soma-Field analogues.

Each row is a single mathematical entity in two different notation systems. The correspondences are not approximate analogies – they are exact identifications under the Wick rotation and the Hopfield equivalence.

GOING DEEPER: The Baez–Lauda Coherence Theorem

In 2011, John Baez and Aaron Lauda proved a coherence theorem establishing that string diagrams are a complete and sound notation for morphisms in symmetric monoidal categories. This means: anything you can write as a morphism in a symmetric monoidal category, you can draw as a string diagram, and vice versa, with perfect fidelity.

Feynman diagrams are string diagrams for the symmetric monoidal category of representations of the Poincaré group (the symmetry group of spacetime). Tensor network diagrams (used in quantum information and condensed matter) are string diagrams for the same structure.

The soma-field operations — emotional mode coupling, field composition, state tensor products — are morphisms in a symmetric monoidal category. Therefore, they can be drawn as string diagrams. Therefore, they can be computed with the same diagrammatic calculus as Feynman diagrams.

This is not the claim that emotions are quantum mechanical. It is the claim that the mathematics of composition and coupling is universal — it appears wherever things interact, regardless of what the things are.

KEY TERMS

Wick rotation — the substitution $t \rightarrow -i\tau$ that transforms oscillatory quantum dynamics into real-time thermal/stochastic dynamics.

Feynman diagram — a diagrammatic notation for computing interaction amplitudes in quantum field theory; each diagram represents a specific integral contribution to a physical quantity.

String diagram — a diagrammatic notation for morphisms in a symmetric monoidal category; identical in structure to Feynman diagrams under the Baez–Lauda theorem.

Morphism — a structure-preserving map between objects in a category; the general notion that subsumes functions, linear maps, and physical interactions.

Chapter 8: The Nervous System as Phase Diagram

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- What phase transitions are and why they apply to the nervous system
 - How the three polyvagal states correspond to different phases
 - Why state changes in trauma feel sudden rather than gradual
 - What ADHD represents in thermodynamic terms
-

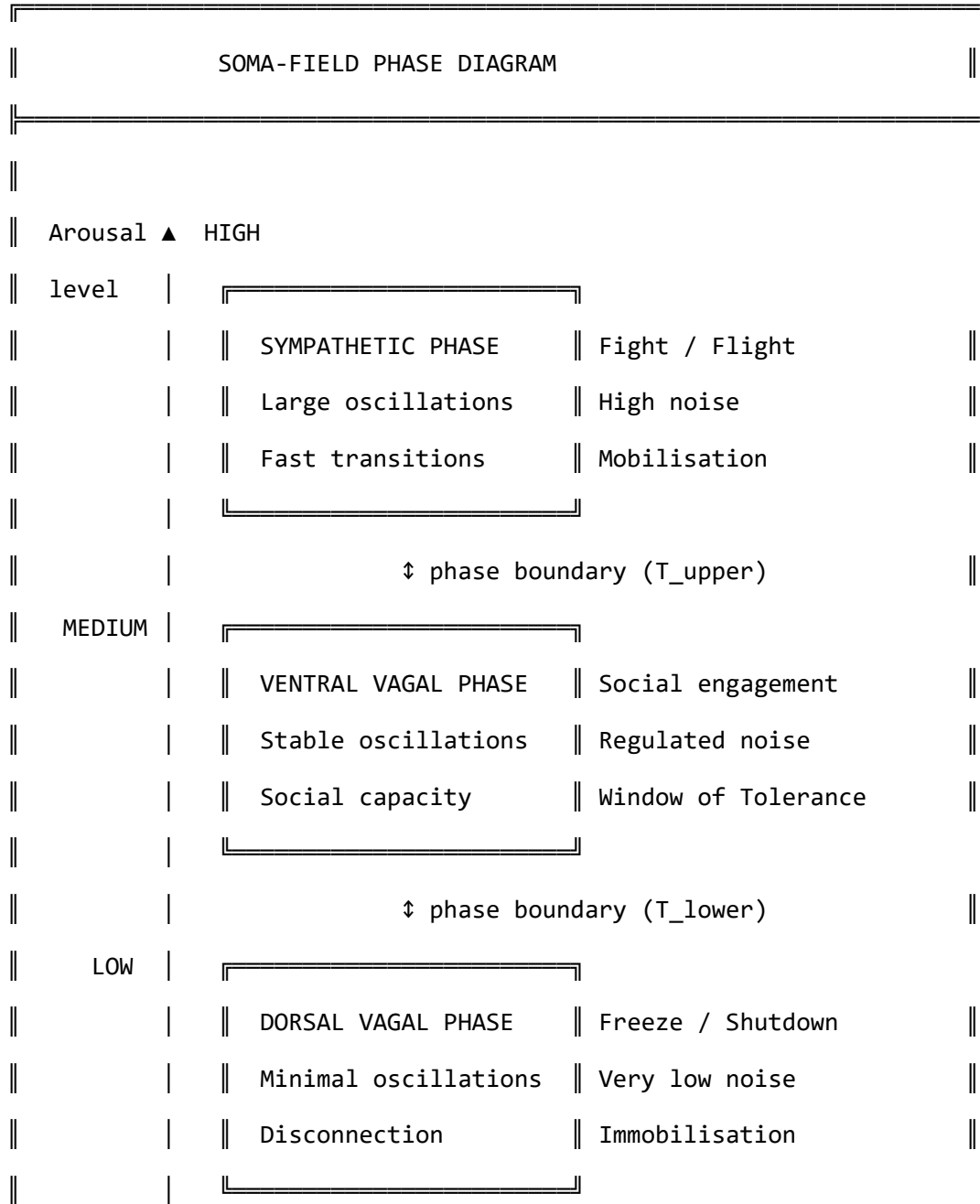
8.1 Phase Transitions

Water can exist as ice, liquid, or steam. At atmospheric pressure, it transitions between these phases at specific temperatures: 0°C and 100°C. The transitions are dramatic: adding energy to ice below 0°C changes its temperature gradually; adding energy at exactly 0°C produces no temperature change — the energy goes entirely into breaking the crystal lattice, reorganising water molecules from a rigid ordered structure into a fluid disordered one. This is a **phase transition**: a qualitative reorganisation of the system's structure at a critical point, rather than a smooth gradual change.

Phase transitions appear wherever there is an energy landscape with multiple stable phases, and a parameter (temperature, pressure, magnetic field) that shifts the relative stability of those phases. They are universal.

8.2 The Three Phases of the Nervous System

The polyvagal hierarchy describes three functional states of the autonomic nervous system. In the Soma-Field Model, these correspond to three distinct phases of the field:



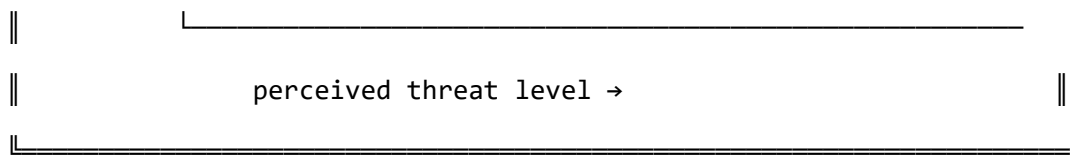


Figure 8.1. The nervous system as a phase diagram. Three distinct phases correspond to the three polyvagal states. Phase boundaries (T_{upper} and T_{lower}) mark the transitions. For a regulated nervous system, most experience occurs in the ventral vagal phase. For a trauma-modified system, the lower boundary T_{lower} may be close to the ventral vagal resting state, making the transition to freeze easier to trigger.

The critical feature of a phase transition — as opposed to a smooth change in arousal level — is that it happens *all at once*. Below the phase boundary, adding arousal increases activation level. At the phase boundary, the system tips: a qualitatively different organisation takes over. This is why the freeze response (dorsal vagal) is not “very very calm”: it is a different phase with different physical properties, entered through a phase transition, not reached by gradual reduction.

This also explains why clients in therapy sometimes describe state changes as happening without warning: from their perspective, they were fine, and then suddenly they were not. From the model’s perspective, they were gradually approaching a phase boundary, and the transition happened when they crossed it. The discontinuity is real — it is a property of the phase diagram, not a failure of self-awareness.

8.3 ADHD: A Thermodynamic Framing

Attention Deficit Hyperactivity Disorder (ADHD) presents quite differently from C-PTSD in the soma-field model. Rather than a modification of the coupling matrix structure, ADHD corresponds primarily to an increase in the **effective noise amplitude** σ_0 and a reduction in **damping** γ of the field dynamics.

The Langevin equation with these parameters:

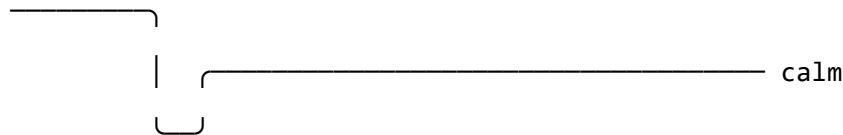
$$\dot{\mathbf{e}} = -\gamma \nabla H(\mathbf{e}) + \sigma_0 \eta(t)$$

In the ADHD regime, σ_0 is large and γ is small. The implications:

- The field moves around the landscape quickly (high noise, low damping)
- It spends less time in any single attractor (shallow dwell time in all basins)
- Transitions between states are frequent and sometimes erratic
- The effective “temperature” of the system is high: many states are thermally accessible

NEUROTYPICAL (moderate σ_0 , moderate γ):

— $\mathbf{e}(t)$: settles at attractor, brief excursions, returns



ADHD (high σ_0 , low γ):

— $\mathbf{e}(t)$: fast, wide excursions, brief attractor dwell

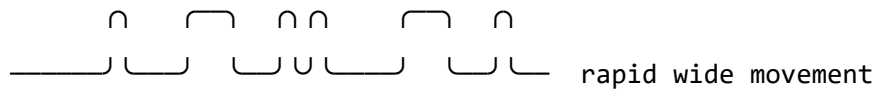


Figure 8.2. Field dynamics in neurotypical (top) and ADHD (bottom) regimes. ADHD is not a broken attractor structure – the landscape may be quite normal. It is a high-temperature, low-damping dynamical regime in which the field moves

through the landscape rapidly and does not settle.

The clinical significance: ADHD is not a motivation or character failure. It is a nervous system running at a thermodynamic setting different from typical, with specific performance characteristics — excellent rapid exploration of large state spaces, poor sustained dwell in narrow regions. “Focus” difficulties arise not because the attractor is absent, but because the effective temperature is too high for the system to remain in it.

The co-occurrence of ADHD and C-PTSD — which is common, and is well-documented — creates a particularly complex landscape: the coupling matrix is asymmetrically modified (C-PTSD effect) *and* the field is running at high temperature (ADHD effect). The practical consequence is a system that has a large, deep hypervigilance attractor and the thermal energy to reach it from almost anywhere.

KEY TERMS

Phase transition — a qualitative reorganisation of a system’s structure at a critical parameter value; not a gradual change but a discontinuous one.

Noise amplitude σ_0 — the magnitude of random fluctuations in the field dynamics; controls the effective temperature of the system.

Damping γ — the rate at which the field returns toward attractor states after perturbation; low damping means slow return.

Effective temperature — the ratio σ_0^2/γ ; determines how widely the field explores the landscape relative to the depth of the attractors.

PART IV: WHAT CHANGES

Chapter 9: The Instrument

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- What the Soma-Field Instrument is designed to measure
 - The seven dimensions the instrument tracks
 - What the ABCD operator circuit does
 - How the instrument relates to clinical practice
-

9.1 The Map Is Not the Territory

The Soma-Field Model is a mathematical description. Like all mathematical descriptions of physical or biological systems, it simplifies. The soma-field is not the body; it is a model of the body, selected for the properties it can illuminate while necessarily omitting others. This is not a failure of the model. A map that included every detail of the territory would be the territory.

The **Soma-Field Instrument** is a clinical tool built on this model: a structured means of tracking the parameters of the soma-field over time — the coupling structure, the attractor positions, the threshold, the noise level, the memory kernel amplitudes — so that changes can be measured rather than merely described.

The instrument is not a questionnaire. It does not ask about narrative or history. It asks about the body: current activation levels across the emotional modes, attractor dwell times, threshold accessibility, interoceptive accuracy. The goal is to make the model's parameters observable.

9.2 The Seven Dimensions

The instrument tracks seven primary dimensions of soma-field state:

THE SEVEN DIMENSIONS OF THE SOMA-FIELD		
1. ACTIVATION LEVEL	How strongly are the modes	
$e = (e_1, \dots, e_n)$	currently firing?	
2. ATTRACTOR POSITION	Which state is the field	
$e^* = \operatorname{argmin} H(e)$	currently resting in?	
3. THRESHOLD	At what activation level does	
T	the field become conscious?	
4. WINDOW OF TOLERANCE	How wide is the basin around	
$\Delta T = T_{\text{upper}} - T_{\text{lower}}$	the current attractor?	
5. NOISE LEVEL	How much thermal fluctuation	
σ_0	is present? (ADHD component)	
6. MEMORY KERNEL AMPLITUDE	How strongly are past	
$A = (A_1, A_2, \dots)$	activations currently echoing?	
7. INTEROCEPTIVE ACCURACY	How reliably can the person	
$\alpha \in [0,1]$	read their own field state?	



Figure 9.1. The seven dimensions of the Soma-Field Instrument. Each dimension corresponds to a parameter or derived quantity of the mathematical model. Clinical progress is tracked as change across these dimensions over time, rather than as narrative self-report alone.

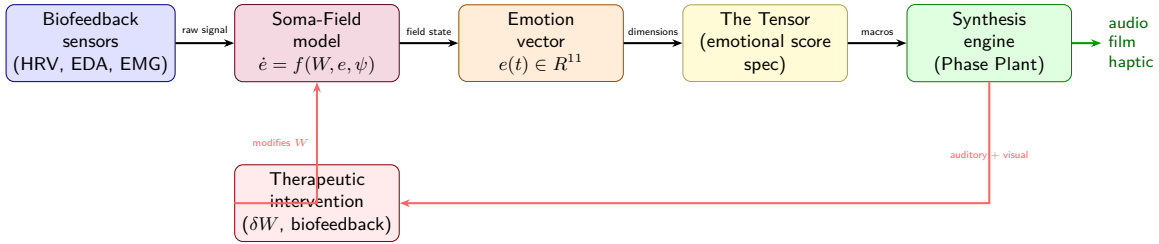


Figure 8: Figure 9.2. The Soma-Field instrument pipeline. Biofeedback sensors (HRV, EDA, EMG) feed the soma-field model, which produces a real-time emotion vector $\mathbf{e}(t) \in R^{11}$. This drives The Tensor (the emotional score specification), which controls a synthesis engine (Phase Plant). A feedback loop via therapeutic intervention W allows the practitioner to modify the coupling matrix directly — closing the loop between measurement and treatment. *Author's original figure.*

9.3 The ABCD Operator Circuit

The instrument is organised around four operators that act on the soma-field:

A — Attention: the operation of directing conscious attention to a body region or emotional mode. Attention modulates the threshold T locally: attended regions have their activation brought closer to or above the threshold. Formally: a projection operator that selects a subspace of the field.

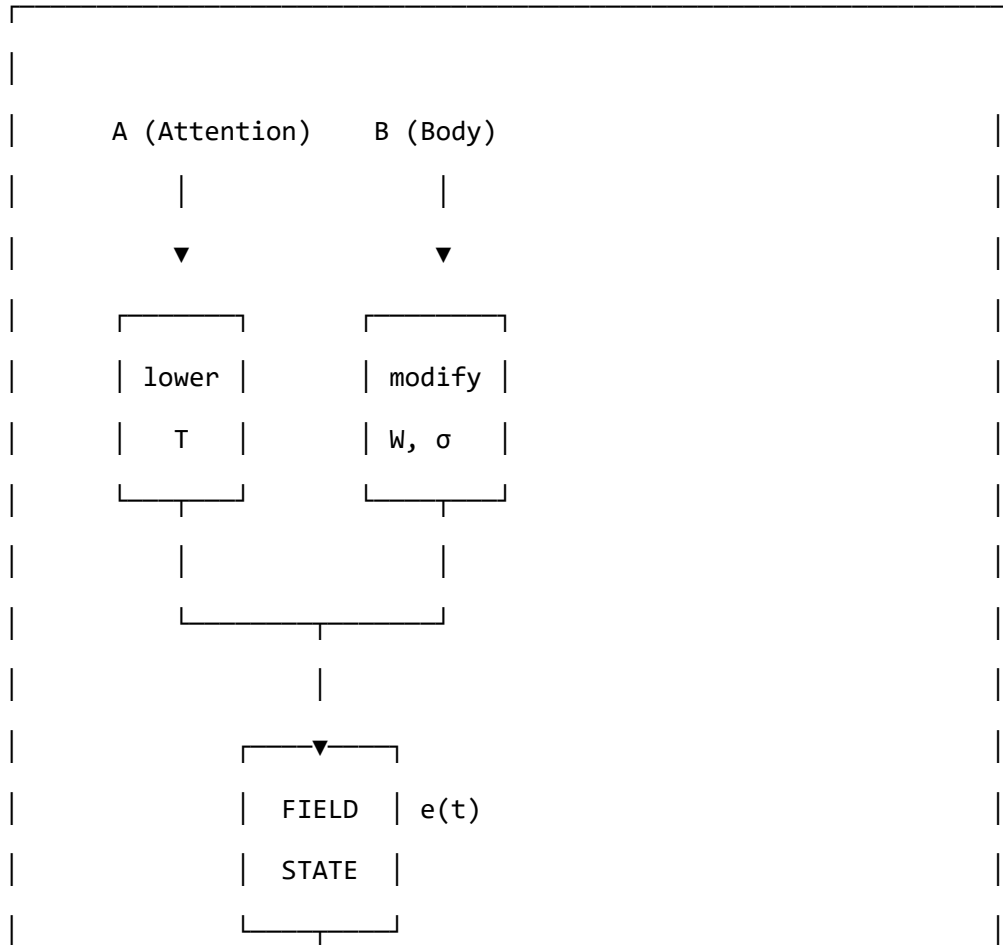
B — Body: the somatic grounding operations — breath, posture, movement, temperature. These directly influence the coupling matrix (changing which modes are activated together) and the noise amplitude (breath regulation reduces σ_0). Formally:

a modification of the W and σ_0 parameters.

C — Coupling: the explicit work of mapping which emotional modes are coupled, how strongly, and in what direction. This is the diagnostic function of the instrument: identifying the current coupling structure so that modifications can be targeted. Formally: an estimation of W from observed field dynamics.

D — Dynamics: tracking field evolution over time — how the state moves, which attractors it visits, how long it dwells, what triggers transitions. This is the longitudinal function: measuring change across sessions.

THE ABCD CIRCUIT



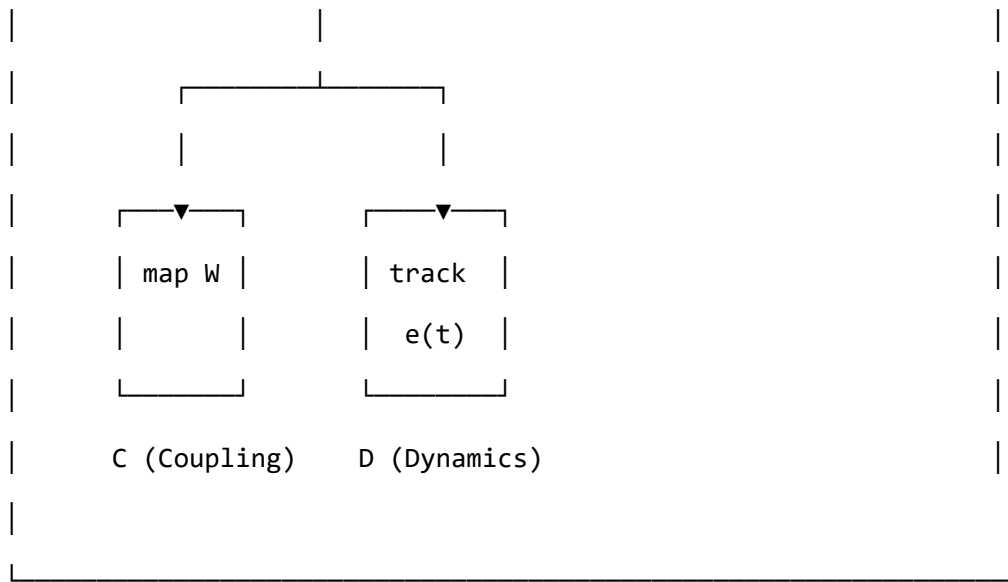


Figure 9.2. The ABCD operator circuit. Attention (A) and Body (B) are input operators that act on the field. Coupling (C) and Dynamics (D) are measurement operators that read from the field. Together they form a closed loop: the measurement informs the input, which modifies the field, which is measured again.

Chapter 10: Forward Transformation

"The opposite of trauma is not safety.

It is a nervous system that can find safety."

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- Why “healing” in the traditional sense is not the right goal for all trauma
 - What forward transformation means in the language of the model
 - What therapy “does” when it works, in terms of field parameters
 - What the new landscape looks like
-

10.1 The Wrong Goal

The dominant model of trauma recovery involves, in some form, a return. Processing the memory until it no longer carries charge. Resolving the dissociated parts. Finding the self that existed before. Returning to baseline.

For late trauma — modification occurring after the baseline is formed — this model is coherent. A baseline exists. The modification can, in principle, be subtracted from the current coupling matrix to recover something close to it. The therapeutic work,

however difficult, is working toward a target that is real.

For pre-verbal trauma, this model generates a problem. The baseline was never fully formed. The target of recovery — the self before the modification — is a mathematical object that does not exist. Attempting to drive the field toward it is attempting to converge on an undefined value.

Clinically, this manifests as therapy that helps, and helps, and helps — and never arrives. Each session improves things. The client gets better at regulation, more tolerant of activation, more able to function. But the destination remains unreachable. The gap persists. The sense of having “a self before all this” that the therapy is trying to restore — never narrows to nothing.

This is not a failure of the therapy or the therapist. It is a consequence of using the wrong map. The destination does not exist; the voyage toward it cannot terminate.

10.2 The Right Goal

Forward transformation changes the question.

Instead of: *how do we remove the modification to recover what was there before?*

We ask: *what kind of coupling matrix W' would give this nervous system the widest possible window of tolerance, the deepest possible calm attractor, and the lowest possible memory kernel amplitudes — starting from where it is now?*

This is a well-posed optimisation problem. W' does not have to be W_0 . It does not have to resemble a neurotypical baseline. It has to have desirable dynamical properties as specified by the clinical goals of this person.

The voyage is not back. It is forward into a landscape that has never existed — a landscape being constructed, not recovered.

THERAPEUTIC TRAJECTORY: FORWARD TRANSFORMATION

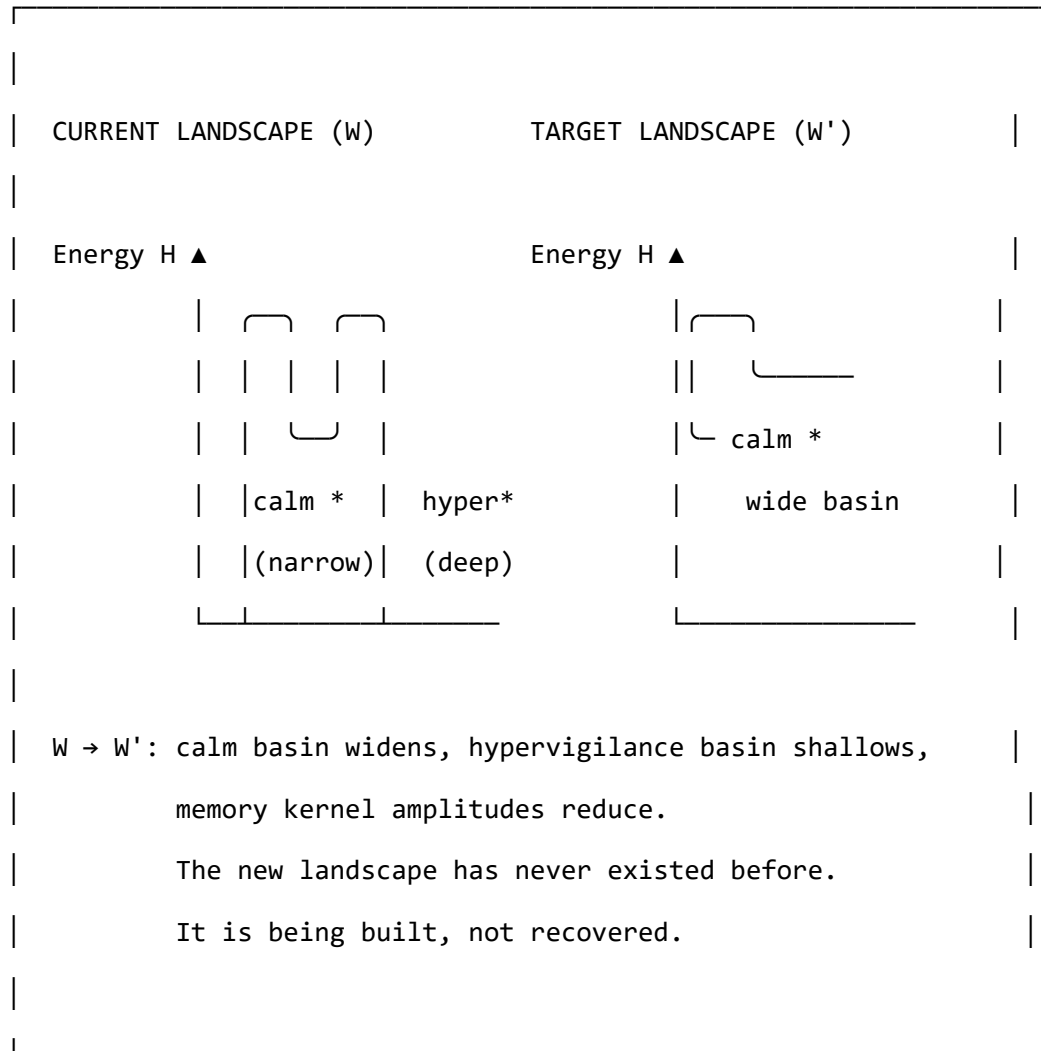


Figure 10.1. Forward transformation. The target W' is not a reconstruction of a prior baseline (which may not have existed). It is a new configuration with desired dynamical properties: a wide calm basin, shallow hypervigilance attractor, and reduced memory kernel amplitudes. The path from W to W' uses therapeutic tools as the mechanism of landscape modification.

10.3 What Therapy Does

In the language of the model, effective somatic therapy for pre-verbal trauma does the following, measurable in terms of the model's parameters:

1. **Widens the window of tolerance** ($T_{\text{upper}} - T_{\text{lower}}$ increases): more activation is tolerable without triggering a phase transition.
2. **Reduces memory kernel amplitudes** (A_k decrease): past activations exert less pull on the current field state. The echoes get quieter.
3. **Increases memory kernel decay times** (τ_k increase): the echoes that remain fade more quickly. The field returns to rest between episodes.
4. **Symmetrises the coupling partially** (W becomes more symmetric): the asymmetric directional flows decrease. Getting from hypervigilance to calm becomes less difficult relative to the reverse journey.
5. **Deepens the calm attractor** (calm basin gets deeper and wider): the field can be perturbed further from rest and still return there.
6. **Improves interoceptive accuracy** (α increases): the person gets better at reading their own field state, which improves the precision of all the above.

None of these changes brings the field to W_0 . All of them make the field W' more functional, more flexible, and more capable of safety. The model does not specify how these changes are achieved — that is the domain of clinical practice. It specifies what is changing when they are achieved.

10.4 The Therapeutic Relationship as Field Coupling

A note on the relational dimension, which the model's formalism can sometimes obscure.

The coupling matrix W is not static. It is updated by experience. The experience of being in a regulated relationship — of having an other whose field is predominantly ventral vagal, engaged, and non-threatening — is itself field-modifying. The nervous system learns from co-regulation.

In field language: the therapist's soma-field is coupled to the client's soma-field during a session. This coupling is weak (they are separate bodies) but not zero. Repeated experiences of this coupling — of another field that is stable and available — gradually shift the client's attractor structure. The calm that is borrowed from the relational field slowly becomes encoded in the client's own coupling matrix.

This is why relational therapy works even in the absence of explicit body-focused techniques. The relationship is the technique. The therapist's regulated nervous system is the instrument.

AUTHOR'S NOTE: The Voyage Forward

I wrote this model in part because I needed a description of my own landscape that was precise enough to work with.

The traditional therapeutic story — you process the trauma, you return to yourself, you heal — did not fit. I got better, session by session, year by year. The regulation improved. The activation windows widened. The freeze responses got shorter. But there was nowhere I was arriving at, no self I was returning to, because the modification had not been added to a prior self. It was the self.

What the model gave me was a different story: not a return, but a construction. Not going back to something, but going forward to something that has never existed. And because the target is W' rather than W_0 , the

voyage does not need to end.

There is no failure in that. There is, in fact, considerable freedom.

KEY TERMS

Forward transformation — the construction of a new coupling matrix W' with desired dynamical properties, as opposed to the recovery of a prior baseline W_0 .

Co-regulation — the process by which the soma-field of one person influences the soma-field of another through relational coupling; the mechanism by which the therapeutic relationship modifies the landscape.

PART V: APPLICATIONS

Chapter 11: A Voyage into the Field

LEARNING OBJECTIVES

By the end of this chapter you should be able to:

- Explain what it means to *navigate* an emotional field rather than merely observe it
- Describe the two sectors of soma-field dynamics: perturbative (within a basin) and non-perturbative (threshold crossings)
- Explain what a Feynman diagram represents and apply the concept informally to emotional coupling
- Define the *emotional score* of a narrative and distinguish it from its container
- Explain the holographic principle as applied to clinical assessment
- Describe what EmotionML captures and what it omits

Two films. One from 1966. One never made.

In *Fantastic Voyage* (Fleischer, 1966), a submarine called the *Proteus* is miniaturised to microscopic scale and injected into the bloodstream of a critically injured scientist. The crew has sixty minutes to navigate from the carotid artery to a blood clot in the brain, dissolve it, and exit before the miniaturisation reverses. The body is the territory. The voyage is literal.

In a therapy session, something similar happens. Attention — the therapist's, and eventually the patient's own — is directed inward. It navigates through layers of somatic sensation, emotional activation, and memory echo. It encounters resistance: the field's own defence against being observed. It approaches regions of high activation

— the deep attractors — and, if conditions permit, crosses the threshold into them rather than turning back.

The body is the territory in both cases. The voyage is into an interior that has its own geography, its own currents, its own immune responses. The *Proteus* crew is attacked by white blood cells — the body’s machinery for destroying foreign objects. The therapeutic attention is resisted by the field’s own homeostatic mechanisms: avoidance, dissociation, intellectualisation, the body’s insistence that some regions remain unvisited.

It was always the same film.

The Soma-Field Model provides the mathematics of that film: the Hamiltonian landscape the crew must navigate, the attractor basins where the submarine drifts without effort, the energy barriers that require thrust to cross, the memory kernel that makes past routes echo in present navigation. This chapter develops the geometry of the voyage.

11.1 The Navigable Landscape

In Chapter 4 we introduced the Hamiltonian $H(\mathbf{e})$ as the energy function of the emotional field. The state of the field is a point in the high-dimensional space of all possible emotional-somatic configurations. The dynamics move the field downhill toward local minima — the attractor basins.

Think of this as terrain. The attractor basins are valleys. The field settles naturally into whichever valley it is closest to, and tends to stay there unless external energy (a trigger, a somatic cue, a therapeutic intervention) pushes it uphill toward a ridge and over into another valley.

The therapeutic voyage is a navigation across this terrain: starting in one valley (the presenting state), moving toward another (the target state — safety, integration, the capacity for contact), crossing the ridges in between. The ridges are the thresholds. The crossing is the therapeutic event.

What the terrain looks like. For a field with two strongly coupled modes — call them *fear* and *shame* — the landscape is a surface in three dimensions: fear on one axis, shame on another, energy on the vertical axis. The attractor is a bowl. The trauma state may have two bowls: a “fear-then-shame” sequence, and a “freeze” attractor from which shame and fear are both absent but unreachable.

For n modes, the terrain is n -dimensional. Visualisation requires projection, but the mathematics is the same regardless of dimension.

Fractal basin boundaries. When the coupling matrix W is asymmetric — when fear drives shame more strongly than shame drives fear, as is common in post-traumatic presentations — the boundary between attractor basins is not a smooth curve. It is fractal: the boundary between the “hypervigilance” basin and the “collapse” basin in a traumatised field has infinitely complex interdigitation at every scale of magnification.

The Mandelbrot set is the mathematical archetype of a fractal basin boundary: the boundary between the set and its complement is a Julia set, infinitely detailed at every scale. The fractal basin boundaries of the soma-field are of the same class. The visualisation is not merely aesthetic — the mathematics is the same.

GOING DEEPER: Fractals, Julia Sets, and Attractor Boundaries

The iteration $z \mapsto z^2 + c$ that generates the Mandelbrot set is a discrete dynamical system on the complex plane. Its attractor is the origin (the sequence converges to 0), or infinity (the sequence escapes). The boundary between the two basins is the Julia set J_c — a fractal object that, for most

values of c , has non-integer (Hausdorff) dimension strictly between 1 and 2.

The soma-field is a continuous dynamical system on $\mathbb{R}^n$, not a discrete iteration on $\mathbb{C}$. But the mechanism is the same: nonlinear coupling between modes (the W_{ij} terms + the threshold nonlinearity) generates sensitivity at the boundary that propagates across scales. The Hausdorff dimension of the boundary is a direct function of the asymmetry of W and the steepness of the threshold nonlinearity. In a severely traumatised field with highly asymmetric coupling, the boundary dimension approaches 2: the boundary is space-filling. There is, in the formal sense, no clean edge between hypervigilance and collapse. Just increasingly complex interdigitation.

Clinical implication. The fractal character of the basin boundary means that small perturbations near the threshold have disproportionate effects — the butterfly effect is concentrated at the boundary. A session conducted near a threshold crossing is qualitatively different from a session conducted well inside a basin. The geometry predicts this before any clinical experience confirms it.

11.2 Emotions Looking for Each Other

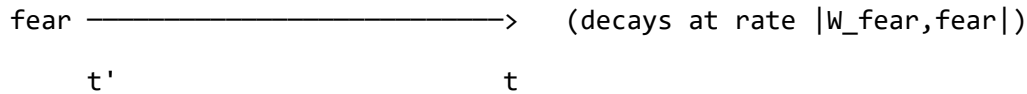
In particle physics, interactions are drawn as Feynman diagrams: lines representing particles moving through space and time, meeting at vertices where something happens. An electron emits a photon. A quark changes flavour. Two particles scatter.

The same formalism applies to the soma-field, and the interpretation is immediate.

The propagator. A single emotional mode — call it *fear* — traveling through time without interacting with anything else is drawn as a single line, moving from left to

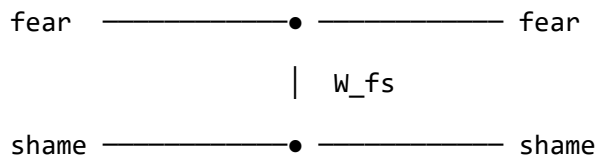
right (earlier to later). The line gets fainter as time increases: the mode decays toward its equilibrium unless maintained by coupling or stimulus. This is the *propagator* — the Green's function of the free dynamics.

Single-mode propagator:



The coupling vertex. When fear and shame are coupled — $W_{\text{fear, shame}} \neq 0$ — they can scatter: fear activates shame, shame amplifies fear. This is drawn as two lines meeting at a vertex, with the coupling strength W_{ij} labeling the junction.

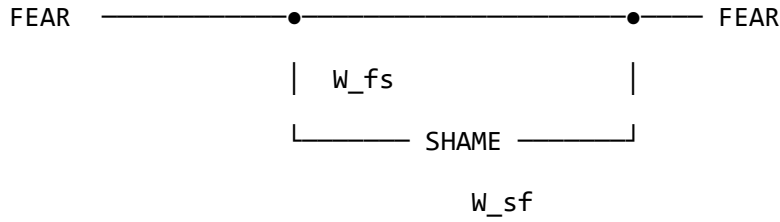
Fear-shame coupling vertex:



If $W_{\text{fear, shame}} > 0$, shame excites fear. If $W_{\text{fear, shame}} < 0$, shame suppresses fear. For the asymmetric case $W_{\text{fear, shame}} \neq W_{\text{shame, fear}}$ — one emotion drives the other more than it is driven in return — the vertex is directional. Fear leads shame in a post-traumatic field. Shame may or may not respond in kind.

Feedback loops. When fear excites shame and shame excites fear in a closed cycle, the diagram is a loop. The loop is not merely a metaphor: it is a precise mathematical object whose value — computed by integrating over the intermediate times — gives a correction to the effective coupling at the loop's characteristic timescale.

Fear-shame feedback loop:



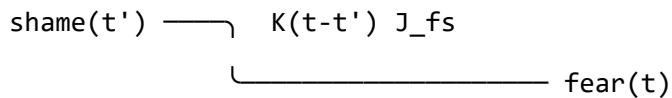
Loop correction: loop runs faster,
effective $W_{fear, fear}$ increases.

This is sensitisation.

Repeated co-activation of fear and shame — as occurs in a trauma where shame was the response to terror — consolidates the loop: the effective coupling grows. The Feynman diagram is a picture of how shame becomes a reliable trigger for fear across sessions and years.

The memory vertex. The trauma memory kernel introduces a vertex that is non-local in time: mode j at some earlier time t' contributes to mode i now, at time t , with weight $K(t - t')$. The diagram has an internal arrow going backward in time — not acausally, but in the sense that the *past state* of the field is still driving the *present state*.

Memory kernel vertex:



The shame at time t' is still driving fear now,
weighted by how much the memory kernel retains it.

For a field with a slow memory kernel (large τ_k), past activations echo far into the future. A traumatic incident twenty years ago is still driving present fear via the memory vertex — not as a belief or a narrative, but as a dynamical coupling with a

specific timescale.

The instanton: the pivot. No finite collection of Feynman diagrams — no sum of scattering and loop corrections — describes a threshold crossing. The topological change from one basin to another is a *non-perturbative* event: an instanton. It is not a series of small steps; it is a qualitative transition, a jump between attractors. In physics, instantons are the events that perturbation theory cannot see. In the therapy room, they are the sessions that change something permanently.

GOING DEEPER: The Two Sectors

Every field theory divides into a *perturbative* sector (small fluctuations, describable by Feynman diagrams) and a *non-perturbative* sector (large topological events, described by instantons, solitons, and other saddle-point solutions).

The soma-field has the same division:

Sector	Events	Mathematical description
Perturbative	Emotional coupling, sensitisation, habituation, day-to-day activation	Feynman diagrams: propagators, vertices, loops
Non-perturbative	Threshold crossings, basin transitions, pivotal sessions	Instantons: minimal-action paths between basins

The perturbative sector is accessible to standard talk therapy (changing W_{ij} by desensitisation; damping memory kernel amplitudes A_k ; adjusting

thresholds). The non-perturbative sector requires conditions for threshold crossing: sufficient activation energy, a safe enough container, and — often — direct somatic engagement. You cannot reach an instanton by accumulating small perturbative steps. That is the formal reason why some therapeutic approaches reach a ceiling.

11.3 The Emotional Score

A musical score is not a performance. It is the abstract structure that can be performed in many ways — by different orchestras, in different halls, at different tempos — while remaining recognisably itself. The *notes* are the invariant; the *sound* is the realisation.

A film has an emotional score. Not the music (though the music is part of its expression), but the trajectory of the emotional field that the film traces over its duration: how activation rises and falls, which modes are coupled, where the thresholds are crossed, what the final attractor state is.

This emotional score is independent of the narrative container — the specific story in which it is realised. The same score can be realised in a river journey, a war, a marriage, a career, a therapy, or a voyage through a bloodstream.

Formally. The emotional score is a trajectory $\mathbf{e}^*(t)$ in the emotional field space, parameterised by story-time $t \in [0, 1]$ (opening to closing). A film, novel, or therapy session is:

$$\text{Realisation} = (\mathbf{e}^*(t), \text{Container})$$

The container provides the narrative surface: characters, setting, imagery, plot. The

emotional score provides the dynamics: which modes activate, in what sequence, at what coupling strength.

The Conrad example. *Heart of Darkness* and its film realisation *Apocalypse Now* (Coppola, 1979) share a score: an upstream journey toward something pre-verbal, toward a figure (*Kurtz*) who represents the field’s deepest attractor — the place where normal threshold regulation has dissolved. The journey upstream is a journey toward decreasing τ_d (shorter developmental time), toward earlier, more diffuse, less differentiated emotional modes. The field becomes less structured as the journey continues. Kurtz is the attractor at the bottom of the developmental basin — not a monster, but the deepest attractor, the one with no threshold above it.

The score is: *progressive reduction of threshold distance, increasing weight of pre-verbal modes, final approach to a basin from which ordinary return is blocked*. The container (Congo river / Vietnam river) is a surface over which this score is played.

Multi-scale structure. The score has fractal structure: the same emotional pattern recurs at the level of the full film, the act, the scene, and the moment. A scene in which a character approaches and retreats from a threshold is a micro-version of the film’s macro-structure. This is not a metaphor — the soma-field dynamics are scale-invariant near a critical point, so the same Hamiltonian structure repeats across timescales. A good filmmaker composes at all scales simultaneously.

The viewer’s field. The viewer has their own emotional field $\mathbf{e}_V(t)$ which couples to the screen signal $S(t)$:

$$\dot{\mathbf{e}}_V = -\nabla H_V(\mathbf{e}_V) + \lambda \cdot S(t) + \eta_V$$

The director controls $S(t)$ — the screen signal — but not H_V (the viewer’s own land-

scape). A viewer whose own field has a deep shame attractor will have a different response to the same $S(t)$ than a viewer without it. The film is the same; the voyage is different. This is the formal account of why films affect different people differently, and why re-watching a film after therapeutic work can produce a qualitatively different emotional experience: H_V has changed.

11.4 The Holographic Clinic

In theoretical physics, the holographic principle (Susskind, 1995; Bousso, 2002) states that the complete description of a volume of space can be encoded on its boundary surface, with no loss of information. A three-dimensional object is fully represented by a two-dimensional hologram. The interior is encoded in the edge.

The soma-field has a holographic structure that is clinically actionable.

The boundary. The observable boundary of the soma-field is what can be seen from outside: behaviour, posture, facial expression, reported affect, the pattern of threshold crossings in session, the rate of escalation and de-escalation, the latency between stimulus and response. This is the boundary data — the hologram.

The bulk. The interior of the soma-field is inaccessible to direct observation: the weight matrix W , the memory kernel $K(\tau)$, the effective thresholds T_i , the attractor topology. These are the bulk fields.

The reconstruction theorem. If the boundary data is sufficiently rich — if we observe enough threshold crossings, enough coupling patterns, enough temporal correlations — the bulk fields can be reconstructed. The weight matrix W_{ij} can be estimated from the co-activation statistics of observed modes. The memory kernel time constants τ_k can be estimated from the delay between stimulus and response at different frequen-

cies. The attractor topology can be inferred from which basins the field visits and how long it dwells in each.

The body tells you everything. This is not a therapeutic truism. It is a measurement theorem: given sufficiently rich boundary data, the full soma-field is recoverable from external observation. The body is a hologram of its own interior.

Clinical implication. A thorough intake assessment — one that tracks not just presented symptoms but response latencies, co-occurrence statistics, threshold distances, and interoceptive access — is a holographic measurement. It gives access to the bulk fields without requiring the patient to verbally report what they do not have words for. The body has been keeping a precise record. The therapist’s task is to read it.

11.5 EmotionML: Labels Without Dynamics

The W3C EmotionML standard (Schröder et al., 2011) provides a formal vocabulary for annotating emotional states in human-computer interaction. It specifies representation formats for emotion categories (anger, fear, joy, sadness...), dimensions (valence, arousal, dominance), and appraisals (novelty, intrinsic pleasantness, goal congruence). It is a well-engineered taxonomy.

It is not a dynamical theory.

EmotionML says what emotional state a system is in at time t . It does not say how that state changes, what coupling it has to other states, what its threshold distance is, how its memory kernel drives its future evolution, or what basin transition conditions apply. It provides a label; the Soma-Field Model provides the dynamics.

The relationship is analogous to the relationship between a chemical nomenclature (naming compounds) and a rate equation (describing how compounds react). The

nomenclature is necessary but not sufficient. Knowing that a patient presents as “fearful” is the EmotionML layer. Knowing the coupling $W_{\text{fear, shame}}$, the threshold T_{fear} , the memory kernel time constants, and the attractor depth is the soma-field layer. The second layer strictly includes the first.

AUTHOR’S NOTE: Why Taxonomy Is Not Enough

The history of psychiatry is largely a history of improving the taxonomy: from humours to syndromes to DSM categories to dimensional models. Each generation’s taxonomy is more precise than the previous. Each is still a taxonomy.

The shift from taxonomy to dynamics is not a refinement. It is a change of mathematical structure: from a set of labels to a vector field on a state space, with a Hamiltonian, a noise term, and a coupling matrix. The prediction capability is qualitatively different. A taxonomy tells you what something is called. A dynamical model tells you what it will do next and what it would take to change it.

EmotionML is a very good taxonomy. The Soma-Field Model is the next layer.

KEY TERMS

Navigable landscape — the Hamiltonian $H(\mathbf{e})$ understood as terrain, with attractor basins as valleys and thresholds as ridges that must be crossed.

Fractal basin boundary — the boundary between attractor basins when the coupling matrix W is asymmetric; has non-integer Hausdorff dimension

and is sensitive to perturbation at all scales.

Feynman diagram — a graphical notation for the terms in a perturbative expansion; in the soma-field, lines represent propagating modes, vertices represent couplings, and loops represent feedback cycles.

Perturbative sector — dynamics within an attractor basin, describable by Feynman diagrams; accessible to standard desensitisation and coupling-modification approaches.

Non-perturbative sector — threshold crossings and basin transitions; described by instantons; requires conditions for the full threshold-crossing event.

Instanton — a non-perturbative saddle-point solution connecting two attractor basins; in the therapy room, the pivot moment that changes the field topology.

Emotional score — the trajectory $\mathbf{e}^*(t)$ that defines a narrative's emotional structure independently of its container; formally $\text{Realisation} = (\mathbf{e}^*(t), \text{Container})$.

Holographic principle — the claim that boundary observables (behaviour, symptoms, threshold patterns) encode the full bulk fields (W , K , attractor topology); the basis for clinical assessment as measurement.

EmotionML — W3C standard for emotion annotation; provides taxonomy (labels) but not dynamics (evolution equations); a necessary but not sufficient layer.

CHAPTER SUMMARY

The Soma-Field Model provides the geometry of a voyage. The Hamiltonian landscape is the territory: attractor basins are valleys, thresholds are ridges, and the field navigates this terrain continuously. For asymmetric coupling matrices, the basin boundaries are fractal — infinitely complex at every scale, sensitive to perturbation everywhere along the edge.

The dynamics divide into two sectors. The perturbative sector — small fluctuations within a basin — is organised by Feynman diagrams: propagators carry modes through time, coupling vertices describe interactions between modes, feedback loops describe sensitisation, and memory kernel vertices describe the echo of past activations into the present. The non-perturbative sector — threshold crossings — is described by instantons: the minimal-action paths between basins that no perturbative sum can reach.

Narratives have emotional scores: trajectories $\mathbf{e}^*(t)$ that define a film or story independently of its narrative container. The same score can be realised in a river journey, a war, or a voyage through a bloodstream. The viewer’s own soma-field couples to the screen signal; what they experience depends on their own Hamiltonian.

The boundary of the soma-field encodes the bulk: behavioural observation, response latency, co-activation statistics, and threshold patterns give access to the full weight matrix, memory kernel, and attractor topology without requiring verbal report of what has no words. The body is a hologram of its own interior.

EmotionML provides the taxonomy. The Soma-Field Model provides the dynamics. Both are needed; the second strictly extends the first.

Epilogue: The T's

There are four T's in this book, and they are not accidental.

Theory — the formal structure that makes prediction possible. A theory is not a guess or an opinion. It is a precise description that can be tested, that makes specific predictions, and that says exactly what evidence would falsify it. The Soma-Field Model is a theory of emotional dynamics: it makes predictions about attractor structure, about the character of pre-verbal versus late trauma, about what parameters change in effective therapy. Whether those predictions survive contact with data is an empirical question, and the empirical work is needed.

Threshold — the parameter T , which appears in the model as the boundary between sub-threshold somatic activity and conscious emotional experience. The threshold is not a switch. It is a continuous parameter, differently set in different nervous systems, modifiable through practice and therapy. The difference between a body that feels everything and a body that feels nothing is, in formal terms, a difference in T . The therapeutic expansion of the window of tolerance is, in formal terms, a widening of the range around T within which the field can move without triggering a phase transition.

Time — the developmental time τ_d , which changes the character of a modification from perturbative (late trauma: $W = W_0 + \delta W$, a baseline plus a modification) to structural (pre-verbal trauma: $W = W_{\text{trauma}}$, the modification is the structure). Time also appears in τ_k — the decay time of the memory kernel, how long an echo persists. Therapy changes τ_k . The passage of time, in the absence of intervention, does not reliably change τ_k for pre-verbal somatic traces.

Transformation — the fourth T, the one that this book is about, finally. Not recovery. Not return. Not the restoration of a prior state. The construction of a new landscape: wider, more flexible, with deeper calm and shallower hypervigilance, arrived at from

where the system is, going somewhere it has not been before.

There is a fifth T, which gave this book its title: **Trance** — in two senses. The first: the altered state at the threshold, the phase transition, the field crossing a boundary and remaining in the other phase. Trance is not a malfunction; it is a dynamic state, momentarily ungoverned by the usual attractors. In those moments, something is possible that is not possible from within a stable phase.

The second sense is the title itself. *A Voyage into Trance* (1995) is a Goa trance compilation by Paul Oakenfold. The trance state produced by extended rhythmic music and the freeze response of a traumatised nervous system are not the same experience. They are governed by the same mathematics. Both drive an arousal variable across a threshold and sustain it there. Music does this deliberately; trauma does it involuntarily. The mathematics does not distinguish.

The voyage into trauma is not a straight line. It passes through all five T's, sometimes in order, sometimes not.

The model is the map. The body is the territory. The voyage is yours.

Appendices

Appendix A: The Mathematics in Full

The following is a condensed version of the academic paper Soma-Field Model of Emotional Dynamics and C-PTSD for readers who want the formal derivations. Full derivations, Lean 4 type sketches, and bibliography are in the companion document `soma-field-paper.md`.

A.1 The Hamiltonian

$$H(\mathbf{e}) = -\frac{1}{2} \mathbf{e}^\top W \mathbf{e} - \theta^\top \mathbf{e}$$

where $W \in \mathbb{R}^{n \times n}$ is the coupling matrix and $\theta \in \mathbb{R}^n$ is the threshold bias vector.

A.2 The Dynamics

$$\dot{\mathbf{e}}(t) = -\nabla H(\mathbf{e}) + \sigma_0 \eta(t) = W \mathbf{e} + \theta + \sigma_0 \eta(t)$$

where $\eta(t)$ is white noise with $\langle \eta_i(t) \eta_j(s) \rangle = \delta_{ij} \delta(t - s)$.

A.3 The C-PTSD Modification

$$W_{\text{C-PTSD}} = W_0 + \Delta W, \quad \Delta W_{ij} \neq \Delta W_{ji}$$

The asymmetry of ΔW breaks the gradient flow property and introduces directional cycles in the landscape.

A.4 The Memory Kernel

$$\dot{\mathbf{e}}(t) = -\nabla H(\mathbf{e}) + \int_0^t K(t-s) \mathbf{e}(s) ds + \sigma_0 \eta(t)$$

$$K(\tau) = \sum_k A_k e^{-\tau/\tau_k}$$

A.5 Developmental Time Parameterisation

$$W(\tau_d) = f(\tau_d) \cdot W_0 + (1 - f(\tau_d)) \cdot W_{\text{trauma}}$$

$$f(\tau_d) = \tanh\left(\frac{\tau_d}{\tau_c}\right), \quad \tau_c \approx 36 \text{ months}$$

A.6 The QFT Correspondence

Under the Wick rotation $t \rightarrow -i\tau$:

QFT	Soma-Field
$G_E(\tau) = \frac{1}{2m} e^{-m \tau }$	$K(\tau) = \sum_k A_k e^{- \tau /\tau_k}$
Field mass m	Inverse decay time $1/\tau_k$
H_{Ising}	H_{soma}

Appendix B: Lean 4 Type Sketches

The following are proof sketches in Lean 4. `sorry` marks open proof obligations.

```
-- Core soma-field structures

structure CouplingMatrix (n : ℕ) where
  W : Matrix (Fin n) (Fin n) ℝ
```

```

     $\theta$  : Fin n  $\rightarrow$   $\mathbb{R}$ 

structure SomaField (n :  $\mathbb{N}$ ) where
    e      : Fin n  $\rightarrow$   $\mathbb{R}$       -- current activation vector
    W      : CouplingMatrix n
    T      :  $\mathbb{R}$                 -- threshold parameter
    sigma  :  $\mathbb{R}$                 -- noise amplitude

-- The Hamiltonian
noncomputable def hamiltonian {n :  $\mathbb{N}$ } (W : CouplingMatrix n) (e : Fin n  $\rightarrow$   $\mathbb{R}$ ) :  $\mathbb{R}$  :=
    -0.5 * Matrix.dotProduct (Matrix.mulVec W.W e) e
    - Matrix.dotProduct W. $\theta$  e

-- C-PTSD modification: asymmetric coupling
def isCPTSDModified {n :  $\mathbb{N}$ } (W : CouplingMatrix n) : Prop :=
     $\exists$  i j, W.W i j  $\neq$  W.W j i

-- Developmental time parameterisation
structure TraumaProfile (n :  $\mathbb{N}$ ) where
     $\tau_d$       :  $\mathbb{R}$ 
    asymmetry   : Matrix (Fin n) (Fin n)  $\mathbb{R}$ 
    amplitudes  : List (Fin n  $\rightarrow$   $\mathbb{R}$ )
    decayTimes  : List (Fin n  $\rightarrow$   $\mathbb{R}$ )

def  $\tau_c$  :  $\mathbb{R}$  := 36

noncomputable def structuralFraction ( $\tau_d$  :  $\mathbb{R}$ ) :  $\mathbb{R}$  :=

```

```

Real.tanh (τ_d / τ_c)

-- For pre-verbal trauma: structural fraction < tanh(1) ≈ 0.76
theorem preVerbalIsStructural {n : ℕ} (profile : TraumaProfile n)
  (h : profile.τ_d < τ_c) :
  structuralFraction profile.τ_d < Real.tanh 1 := by
  unfold structuralFraction
  apply Real.tanh_lt_tanh
  exact div_lt_one_of_lt h (by norm_num)

```

Appendix C: The Cross-Language Correspondence Table

Mathematical language	Emotional dynamics
Symmetric monoidal category $\mathcal{C}$	Soma-field operator algebra
Object $A \in \mathcal{C}$	Emotional mode type
Morphism $f : A \rightarrow B$	Field operator (maps one mode to another)
Tensor product $A \otimes B$	Simultaneous activation of modes A and B
Composition $g \circ f$	Sequential field operations
Identity morphism id_A	Identity (mode persists unchanged)
Braiding $\sigma : A \otimes B \cong B \otimes A$	Mode-order independence of simultaneous states
Feynman vertex	Emotional interaction (coupling W_{ij})
Loop diagram	Memory kernel (self-coupling over time)
Feynman propagator	Memory trace decay $e^{- \tau /\tau_k}$

Mathematical language	Emotional dynamics
Vacuum state	Resting soma-field (minimal activation)
Partition function Z	Field normalisation (probability distribution over states)
Renormalisation group flow	Therapeutic modification of coupling constants
Phase transition	Polyvagal state transition (ventral/sympathetic/dorsal)
Symmetry breaking	Asymmetric W (C-PTSD modification)

The correspondences in this table are not analogies. They are identifications of the same mathematical object in two notation systems, established by the Baez–Lauda coherence theorem (2011) for the categorical column and the Wick rotation for the QFT column.

Appendix D: Glossary

Amplitude A_k — The strength of a trauma memory trace’s influence on the current soma-field. Reduced by effective somatic therapy.

Attractor — A stable state in the energy landscape; a position toward which the soma-field naturally moves from nearby states.

Basin of attraction — The region of state space from which the field flows toward a given attractor.

C-PTSD operator — The modification ΔW to the coupling matrix that represents the effect of complex developmental trauma on the soma-field landscape.

Co-regulation — The process by which the soma-field of one person influences another through relational coupling; the somatic mechanism of relational healing.

Coupling matrix W — The matrix encoding the interactions between emotional modes; determines the shape of the energy landscape. Symmetry of W guarantees stable attractors.

Damping γ — The rate at which the field returns toward attractors after perturbation. Low damping (ADHD) produces rapid, wide-ranging field dynamics.

Decay time τ_k — How long a trauma memory trace persists before fading. Pre-verbal traces typically have longer decay times.

Developmental age at trauma τ_d — The age in months at which the primary traumatic modification occurred. Determines whether the modification is perturbative (late trauma) or structural (pre-verbal trauma).

Effective temperature — The ratio σ_0^2/γ ; determines how widely the soma-field explores the landscape relative to the depth of the attractors.

Forward transformation — The construction of a new coupling matrix W' with desired dynamical properties, as the correct therapeutic goal for pre-verbal trauma (as opposed to recovery of a prior baseline).

Hamiltonian H — The energy function that assigns a value to every possible soma-field state; determines the landscape's hills and valleys and thus the dynamics.

Interoception — The nervous system's process of sensing the body's internal state.

Interoceptive accuracy α — The precision with which a person can read their own soma-field state. Disrupted by trauma; improvable through training and therapy.

Memory kernel $K(\tau)$ — The function describing how past field activations continue to influence the current state. In C-PTSD: a sum of decaying exponentials.

Noise amplitude σ_0 — The magnitude of random fluctuations in field dynamics. Elevated in ADHD; reduced by breath and autonomic regulation.

Phase transition — A qualitative reorganisation of the field’s state at a critical parameter value. Polyvagal state changes (e.g., ventral vagal to freeze) are phase transitions, not gradual changes.

Soma — The body as experienced from the inside; the totality of interoceptive signals.

Soma-field — The vector $\mathbf{e}$ of somatic activation levels across emotional modes; the state of the body’s emotional field at a given moment.

Structural fraction $f(\tau_d)$ — The proportion of the coupling matrix attributable to neurotypical baseline development versus trauma-formed modification.

Threshold T — The activation level above which a soma-field mode becomes conscious experience. The boundary between felt emotion and sub-threshold somatic activation.

Verbal encoding threshold τ_c — The approximate developmental age (36 months) at which narrative memory and verbal encoding capacity reliably emerges.

Wick rotation — The substitution $t \rightarrow -i\tau$ that transforms oscillatory quantum dynamics into real-time thermal/stochastic dynamics; the bridge connecting QFT propagators to soma-field memory kernels.

Window of Tolerance — The range of arousal within which the nervous system can function flexibly, process information, and engage socially.

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Listening Notes

This book was written in a single session on the night of 16–17 May 2026.

The development was set to *Silver Machine* by Hawkwind. It closed with *It's So Easy* by Hawkwind.

Both choices were correct.